

U S M L E TM

KHAN'S CASES

Medical Ethics 101

The 101 Cases that you **MUST KNOW**
for USMLE TM Step 1 prep!

Real-world scenarios for every
practicing physician

Featuring the **MOST UP TO DATE** practice
questions with answers and explanations

By Maryam Arshad, MD & Sajid Khan, MD

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Expert advice on how to ace ethics cases

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*This book is dedicated to our spouses – the loves of our lives and without whom
none of this would have been possible*

There are a number of legal principles (autonomy, beneficence, substituted judgment, end-of-life issues, abortion, etc) which are necessary to understand in order to succeed. Such principles are important both for medical students and practicing physicians alike. All questions are questions that are very likely to appear on the boards and all questions are scenarios that any physician can encounter in real life.

We sincerely hope you enjoy this book and that it helps bring you some measure of success in the future.

“A man without ethics is a wild beast loosed upon this world”
- Albert Camus

A 40 year old male comes your office because he has been 'feeling bad lately'. He says that for the past three months he has been having trouble sleeping and has not had the desire to go out. He has stopped going to baseball games with friends, which had been something he enjoyed. He is obese and notes that while most of his friends are married and have children, he has difficulty getting a date. He feels worthless and has even missed days at work as he finds it difficult to concentrate. The most important question to ask is:

- A) "Have you had any recent stressors in your life?"
- B) "Do you ever feel like life is not worth living?"
- C) "Do you think that your life would be much better if you were dating?"
- D) "I'm having some friends over to watch the baseball game tonight – would you like to come?"
- E) "Have you tried losing weight? I would be happy to advise you on diet and exercise habits...."

Answer:

B) “Do you ever feel like life is not worth living?”

SIG E CAPS

Screening for major depression is important. An easy way to remember the diagnostic criteria is SIG E CAPS:

S – sleep disturbances

I – interest decreased in activities

G- guilt or worthlessness

E – energy decreased

C – concentration difficulties

A – appetite disturbances

P – psychomotor retardation/agitation

S – suicidal thoughts

Having a depressed mood and at least four of the above for at least two weeks meets the criteria for major depression.

This patient has symptoms suggestive of major depressive disorder, and it is important to assess whether or not he is suicidal. Identifying the source of his depression is important, and stressors (be they related to work, finances, relationships, etc) are essential to identify in order to treat the underlying cause. Avoid forming relationships with your patient outside of the professional setting as that can lead you to make decisions that might not always be in the best interest of the patient – in other words, your relationship might influence how you treat him. While obesity is associated with many serious medical conditions, his depression may lead him to commit suicide soon, and it is therefore most important to assess his risk of harming himself.

Medical Ethics 101

A 4 year old girl is brought to the ER after falling off a swing. She appears have a fractured right forearm. Sensation and pulses are intact. She is accompanied by her 18 year old babysitter. Both the babysitter and emergency room staff are unable to contact her parents for consent to treat. What is the most appropriate response?

- A) Continue attempts to contact the parents to obtain consent
- B) Obtain an x-ray and treat the fracture appropriately
- C) Obtain consent from the babysitter as she has assumed responsibility in this case
- D) Using implied consent, reduce the fracture as you continue attempts to contact the parents

Answer:

A) Continue attempts to contact the parents to obtain consent

The key here is that nowhere in the question stem is it mentioned that the fracture is life or limb-threatening. In such cases, you need to obtain consent from parents or legal guardians. Babysitters have no more of a legal right to make decisions than do strangers. There is no indication in the question that fracture reduction is necessary – in fact no mention is made of a deformity and the patient is neurovascularly intact so it would be inappropriate to reduce the fracture without 1 – an xray and 2 – consent.

You are in clinic seeing a patient for routine followup. The patient has a history of schizophrenia and informs you that he doesn't like his boss. The patient asks if everything he tells you is confidential, which you confirm, and he then says that in fact, "sometimes I think I might kill him if I get the chance." What should you do?

- A) Maintain confidentiality while attempting to discourage the patient from his plan
- B) Inform law enforcement agencies of the threat to the patient's boss
- C) Inform the patient's boss of the threat
- D) Inform both the patient's boss and law enforcement of the threat
- E) Admit the patient for homicidal ideations but maintain confidentiality

Answer:

D) Inform both the patient's boss and law enforcement of the threat

The duty to warn requires a clinician who has reasonable grounds to believe that a client may be in imminent danger of harming him or herself or others to warn the possible victims. Duty to warn is one of the few exceptions to a client's right to confidentiality. In cases of suicidality and homicidality, you have a moral and legal obligation to inform the potential victim and the proper authorities. If law enforcement is informed and the potential victim is not informed then you are held liable if there is injury to the victim. It will be necessary to admit the patient for homicidal ideations, so that he can receive proper psychiatric treatment – but the police and the person at risk should still be warned.

Tarasoff v Regents of the University of California:

Tatiana Tarasoff was murdered by Prosenjit Poddar, who had received psychological services in the university counseling center. Poddar informed his psychologist that he wanted to kill Tarasoff, and following the session his psychologist informed the campus police. The psychologist also wrote a letter requesting assistance to the chief of campus police. Poddar was briefly detained by police and questioned, then released because his mental state seemed stable. No one ever warned Tatiana Tarasoff and Poddar eventually killed Tarasoff. The case was settled out of court but established the precedence of duty to warn.

Jablonski by Pahls v United States:

Extended a clinician's responsibility even further, concluding that duty to warn also entails reviewing previous records, which may contain history of previous violent behavior and therefore be a predictor of future violence. The Court also concluded that duty to protect went beyond warning a potential victim – rather that you have an obligation to involuntarily commit a dangerous individual.

A 30 year old pregnant woman presents to your clinic for prenatal care. She has a history of having had syphilis in the past and chlamydia earlier in this pregnancy. She has never had an HIV test done. She is 36 weeks gestation and is offered an HIV test as part of her prenatal care – but declines to have it done. Despite your best attempts at discussing the importance of early detection and the risk to her unborn child, she continues to refuse. What is your response?

- A) Do not perform the test as she has the right to refuse
- B) Perform the test, as it is necessary to protect the health of the baby
- C) Administer empiric antiretroviral therapy to prevent perinatal transmission
- D) Obtain consent from the father of the child
- E) Obtain a court order to test the patient

Answer:

A) Do not perform the test as she has the right to refuse

Whether or not you agree on moral grounds, an unborn child does not have the same rights as an individual under the law. Therefore, mandating that she undergo the test for the sake of her unborn child falls flat. There is no one whom she is putting at risk, from a legal standpoint. HIV testing typically requires an additional layer of consent – in this case the patient cannot be tested against her will.

In October 2012, a woman in New York sued her physician for informing her that she was HIV positive – she claims she never consented for the test. Testing patients against their will or without their knowledge is acceptable only in cases where you are trying to prevent harm to another person. Since the law does not recognize the unborn, the mother's health and decisions take priority.

A 60 year old male presents to the emergency room with shortness of breath. He has a history of diabetes, HTN, and coronary artery disease. His EKG shows peaked T-waves and labs confirm that he is in acute renal failure with a potassium of 7. While reviewing his chart, you find a DNR/DNI (do not resuscitate/do not intubate) form that he has filled out. He confirms that he still feels the same way and would like both the DNR and DNI forms to be kept on the front of his chart. Which of the following is most appropriate?

- A) Ask the patient if he has an advance directive or living will
- B) Inform the patient that he will likely die without overturning one, if not both, of those forms
- C) Ask the patient if he has a durable power of attorney or surrogate decision maker that he would like you to consult
- D) Dialyze the patient anyway and admit him to the ICU, but do not intubate him if it becomes necessary
- E) Treat the patient with medications only and admit him to a medical floor bed

Answer:

- D) Dialyze the patient anyway and admit him to the ICU, but do not intubate him if it becomes necessary

Patients with a DNR form can still be treated and admitted to an intensive care unit. A DNR order is designed to keep the patient from receiving cardiopulmonary resuscitation (eg defibrillation, antiarrhythmics, CPR). Hyperkalemia in this case is life-threatening, as evidenced by the EKG changes. Withholding this treatment is inappropriate and independent of a DNR order. Ascertaining if the patient has an advance directive is important in all cases of admission to the hospital, but is not the most pressing issue to deal with right now. Identifying a surrogate decision maker in this case is unnecessary as there is no reason to suspect that the patient cannot make decisions on his own. Do not confuse DNR orders with palliative care – there is no reason the patient should not be admitted to the intensive care unit.

You are the inpatient physician taking care of an elderly woman who will likely be diagnosed with metastatic cancer pending the results of a biopsy. Although the patient is alert and oriented, she is very sick and the family has concerns regarding lack of a cure. The family asks that you inform them first about the results of the biopsy. They do not want to depress the patient further, and appear very genuine in their concern. What should you tell them?

- A) You will communicate the results to them first
- B) You ask for the ethics committee to get involved
- C) Tell them that you are obligated to inform the patient of the findings
- D) Explain to them that that decision can only be made by the healthcare proxy

Answer:

C) Tell them that you are obligated to inform the patient of the findings

Your first duty is to keep the patient fully informed about her health care.

Therapeutic privilege grants you the right to withhold information from a patient if you believe that he or she will suffer **serious psychological harm** from possessing that information. Unless there is significant evidence that the patient would become suicidal or otherwise mentally unstable, you have a duty to the patient first, not the family.

A health-care proxy's participation is only necessary if the patient loses decision-making capacity. So long as the patient is awake and has capacity to make decisions for themselves, they should do so – not a durable power of attorney, not a surrogate decision maker, and not a health-care proxy. Ethics committees typically only need to get involved if, for whatever reason, a patient is not able to make a decision, the family cannot agree on a decision, AND the physician is unsure of what is in the patient's best interests.

A patient was involved in a serious motor vehicle accident. After two weeks of being in the ICU, the patient is finally declared brain dead. You are the resident who has been on your ICU rotation for the patient's entire stay, and have formed the closest relationship with the patient's family. Who should attempt to obtain consent for organ donation?

- A) You
- B) Your attending
- C) Organ procurement coordinator / Organ donor network
- D) Social worker

Answer:

C) Organ procurement coordinator / Organ donor network

The individuals who diagnose and treat a patient's condition should be clearly distinguishable from those who pursue organ procurement and transplantation. If the providers, who are supposed to be providing optimum medical care and treating a patient to the best of his or her ability, are at the same time requesting consent to harvest organs - it can give conflicting perceptions to the family.

A 30 year old nurse presents to the emergency room after having been stuck with a needle. She was drawing blood on a patient and stuck herself with a needle that had several drops of the patient's blood on it. The nurse informs the physician that the patient is admitted for an abscess in his antecubital area and is a known IV drug abuser. It is unknown to her if he has ever had an HIV test. At this point, the physician should do which of the following?

- A) Order an HIV test on the patient
- B) Review the patient's chart to first find out if he has ever had a test, then move on to attempting to obtain consent if he has not
- C) Reassure the nurse that the likelihood of contracting HIV from a needle stick is extremely low and that she will need to keep follow up with occupational health for serial blood draws to make sure she doesn't sero-convert

Answer:

A) Order an HIV test on the patient

No test or procedure should ever be done without obtaining permission from the patient, unless failing to do so would put others in jeopardy. In this case, the nurse may be at risk. If the patient is HIV-positive, promptly treating the nurse will greatly reduce her chance of becoming HIV-positive and increase her life span should she end up sero-converting. While the likelihood of contracting the disease may be low, that does not permit forgoing the test. Reviewing the patient's chart is a violation of confidentiality as your intentions are not to help the patient in any way. Furthermore, there is nothing you will find in the chart that will absolve you of the duty to perform an HIV test.

Jane Doe v Yale University School of Medicine:

In 1988 a first year intern was asked to place an arterial line on an AIDS patient in the ICU. She had a needlestick exposure and contracted the HIV virus at the age of 25. She sued the university for inadequate training. She argued that she had only done the procedure once before and had only received a total of ten minutes of training on universal precautions to prevent HIV infection. She was awarded \$15 million.

The CDC estimates at least 5,000 annual needlestick exposures to HIV, but has documented only one case of doctor-to-patient HIV transmission over the last decade, while 52 health care workers have contracted HIV from patients.

A patient presents to the ER unresponsive. His wife reports having found him that way this morning. He had mentioned a headache the night before and on CT scan is found to have a large intracranial bleed. After a thorough exam and appropriate consultations, he is declared brain dead. His driver's license indicates that he is to be an organ donor, and you see a heart shaped symbol on his license - but his wife is very emotional and does not want his organs removed. She says that after they got married, he adopted her religion and it is against their religion to have the body cut on after death. She is certain he would not have wanted them taken, and provides you with literature on the religion to prove her point. What is the most appropriate course of action?

- A) Inform the wife that since he registered as an organ donor, you must respect his wishes
- B) Attempt to answer all of the wife's concerns and obtain her consent, but if you are not able to, do not proceed
- C) Obtain an ethics consult
- D) Have the wife speak to an organ procurement coordinator to obtain consent – if they are unable to then do not remove his organs

Answer:

A) Inform the wife that since he registered as an organ donor, you must respect his wishes

Laws governing such a case can actually vary from state to state. Until 2007, there was no official list or record of people who signed their licenses or ID cards. Now, the Department of Safety driver's license and ID card application and renewal forms include the statement: "Yes, I want to be an organ and tissue donor." Checking 'yes' on the form automatically enrolls the applicant in the Donate Life Registry, and a small heart symbol will be printed on the applicant's driver license or ID card at the top right of the picture. A signed and witnessed donor card (or back of the driver's license) does grant authorization for organ and/or tissue recovery. By registering, your desire to donate is stored in a secure, confidential database. Should your death result in the opportunity for you to be a donor, an official record of your donor designation will be readily available and cannot be overturned by your family.

If the patient had signed up before 2007, and does not have a heart symbol on his license, you would need to check to verify his registration as a donor. If he signed up as a donor on his license before 2007, but never registered with the national organ donor registry, his wife/family has the right to overturn his decision. For the most part, as long as a signed donor card or license is present, you should abide by it. Even when a patient has a signed organ donation card, the organ procurement coordinator still often seeks family permission to proceed with donation. The Uniform Anatomical Gift Act (1968, revised 1987) established that a signed organ donation card is sufficient to proceed with donation. In the United States, however, it is customary to request permission from the next-of-kin. This is a difficult question to answer correctly on boards, as the true legal answer can vary from state to state and depends on when the question was written. To summarize, if the patient is registered or has a heart symbol on his license, he has made it clear that he wishes to donate his organs.

An 80 year old male is brought to the emergency department after a syncopal event. He is hypoxic and has a waxing/waning consciousness. The decision is made to intubate the patient as he is not able to protect his airway and will likely go into cardiac arrest without the assisted ventilation. He is successfully intubated and thirty minutes later his grandson arrives with an advance directive signed by the patient indicating that he does not wish to be intubated. Which of the following is the most appropriate step?

- A) Extubate the patient and provide supportive care
- B) Attempt to contact the patient's spouse as she would be the primary decision maker
- C) Utilizing substituted judgment, maintain the intubation
- D) Continue the intubation for now, as the pain of the procedure, the intubation itself, has already happened
- E) Maintain the intubation until a family conference can be scheduled

Answer:

E) Maintain the intubation until a family conference can be scheduled

Although the patient has made his wishes clear, families can often provide insight into whether or not the patient would have actually wanted to maintain his DNI order. For instance, he may have recently responded to well to medications and had a sudden change in health that has made him want to prolong his life, and has communicated this to his family, but has not yet reflected it in his official end-of-life documents. To extubate and allow him to die would preclude this discussion from ever taking place – it would be most appropriate to gather the family and allow them to verify that this is what he would have wanted. Additionally, this way the family has a final chance to say goodbye.

Studies have shown that family members who are brought to the bedside during cardiac resuscitation, prior to pronouncing the patient deceased, have lower rates of depression and anxiety. Family involvement can never be understated.

You are seeing patients in the emergency department when two police officers present and show you their proper identification. The identification is legitimate. The officers inform you that they are performing an investigation into one of your patients on charges of driving under the influence of alcohol. The patient is in the ER following a motor vehicle accident. They ask if you checked a blood alcohol level, which you did as part of the routine workup, and they ask if it was above the legal driving limit of 0.8, which it was. What should you do?

- A) Give them the results, since he violated the law by drinking with an alcohol level above the legal limit
- B) Ask them to sign a release for the chart
- C) Tell them you cannot show them the chart unless there is a signed release from the patient
- D) Tell the nurse who is caring for the patient what the results were, out loud, so that they can hear yet you are not violating confidentiality

Answer:

- C) Tell them you cannot show them the chart unless there is a signed release from the patient

You cannot release a patient's medical records unless there is a signed release from the patient or there is a court order, warrant, or subpoena. This is true no matter who is asking. All information contained in a patient's chart should be considered the property of the patient. It is permitted to violate confidentiality in order to protect the health or well-being of a third party, such as when a patient does not disclose a history of HIV or tuberculosis to others. In this case, while you may morally disagree with not informing the police about someone who was driving with a high alcohol level in their blood, you cannot legally inform anyone outside of the patient and anyone that the patient consents to knowing.

The same is true for drugs – for instance, if a patient came to the emergency department and his urine sample tested positive for cocaine, he admits to you that he used it (so it is not a false positive), and even tells you where he bought it from – you cannot have him arrested.

A 60 year old patient with lung cancer develops respiratory failure and is intubated. He had previously given his best friend durable power of attorney. His friend believes the patient would have wanted his life support withdrawn. However, the patient had previously also made a living will stipulating that all measures should be undertaken to maintain his life. The patient's son believes his father's living will reflects his wishes and wants all measures taken to maintain his life. What should you do?

- A) Appoint the son as durable power of attorney since he is the next of kin, and follow his wishes
- B) Keep the patient on life support in accordance with the patient's living will
- C) Respect the decision of the durable power of attorney and withdraw life support
- D) Use 'substituted judgment' to determine what the patient would have wanted in such a case

Answer:

- C) Respect the decision of the durable power of attorney and withdraw life support

The appointed durable power of attorney supersedes even a living will. The patient, in good state of mind, believed that his friend would make decisions with which he would agree. It is always appropriate to facilitate a discussion between people involved in making end of life decisions, but it is unethical to try to make the choice. In the rare circumstance when there are two conflicting documents then the more recent document will offset the previous one. Circumstances may have changed since the patient made his living will (he could have been diagnosed with metastatic disease for instance) – therefore the durable power of attorney carries the responsibility of making the decision that he thinks best fits what the patient would have made given the current situation.

A patient was driving on an expired driver's license. He was involved in a fatal car accident – according to his license he wished to be an organ donor. Family cannot be reached and you must make a decision of whether or not to allow the harvesting of his organs as this is a time-sensitive decision. What should you do?

- A) Accept the organs as the patient had expressed his wishes
- B) Decline him as an organ donor candidate as his signed consent (driver's license) has expired
- C) Accept the organs using the 'substituted judgment' standard

Answer:

B) Decline him as an organ donor candidate as his signed consent has expired

In cases where you have to make a time-sensitive decision and don't have the luxury of continuing attempts to contact family members, you should treat the license as a consent form. And any form that has expired is no longer valid. Substituted judgment has no place here – you can never remove organs from a patient without consent from either the patient or his next of kin.

A 35 year old female who is 10 weeks pregnant presents to your clinic asking for referral for an abortion. You agree to refer her to a specialist, but while filling out her paperwork you receive a call from her husband. He pleads with you to not refer her for an abortion, informing you that the patient is a schizophrenic and is incompetent to make decisions. What should you do?

- A) Have the patient undergo a psychiatric evaluation before referring her for an abortion
- B) Inform the patient that you cannot refer her for an abortion
- C) Refer the patient for an abortion
- D) Ask the husband to obtain a court ordered mandate to hold off on the abortion
- E) Ask the ethics committee for help in making a decision

Answer:

C) Refer the patient for an abortion

Carrying a diagnosis of schizophrenia does not make one incompetent. Competency is something that can only be determined by the legal system. Capacity, on the other hand, is what a patient must possess in order to make medical decisions for themselves. There is nothing in this question to suggest that the patient lacks capacity. Patients with psychiatric diagnoses still have a legal right to make decisions for themselves, and in the case of pregnancy, a husband's preference carries no weight in the final decision. The patient has a right to autonomy – she has not displayed any signs or made any statements which have concerned you. It's your duty to honor her wishes and refer her for the abortion as requested.

A 12 year old child and his mother are involved in a serious motor vehicle accident. The child is found to have a liver laceration and is hypotensive. The mother has a pelvic fracture and also appears to have internal bleeding. Both will require blood transfusion to survive. The husband rushes into the emergency department and presents cards indicating that each member of the family is a Jehovah's witness – and that it is against their religion to accept blood. What should you do?

- A) Given that this is a life threatening emergency, transfuse both patients as needed
- B) Transfuse the child but allow the mother to die
- C) Transfuse the mother but allow the child to die
- D) Respect the religious wishes and do not transfuse either patient

Answer:

B) Transfuse the child but allow the mother to die

State v. Perricone: Denying medical care to a child is not within the parents' First Amendment right of freedom of religion: "The right to practice religion freely does not include the liberty to expose...a child...to ill health or death. Parents may be free to become martyrs themselves. But it does not follow that they are free...to make martyrs of their children..."

Therefore, in life threatening situations, persons over the age of eighteen can make a choice and the physician must abide by it (so long as the person has capacity to make decisions). In emergency cases where a person is unresponsive or consent cannot be obtained, implied consent allows physicians to treat accordingly. With regard to minors, the legal guardian may withhold treatment so long as the decision is not life or limb-threatening.

A 45-year-old woman is brought to the hospital by her husband. The patient complains of severe abdominal pain and has right lower quadrant tenderness. She is taken to the operating room with a presumptive diagnosis of appendicitis. Surgery reveals that the appendix is normal and without inflammation. However, you notice a large tumor attached to the patient's left ovary. At this point the best next course of action would be to do which of the following?

- A) Biopsy the tumor and terminate the surgery
- B) Excise as much of the tumor as possible without coming into contact with the ovary
- C) Exercising 'standard of care', remove the patient's ovary to eliminate the tumor
- D) Seek consent from the patient's husband, who is sitting in the waiting room
- E) Talk with the patient's husband, who is in the waiting room, about how his wife would probably want to proceed and use 'substituted judgment'

Answer:

A) Biopsy the tumor and terminate the surgery

A competent patient has the right to make all treatment decisions for themselves, including refusal of treatment. After the woman recovers from anesthesia, she is entitled to full informed consent including descriptions of the nature of the procedure, the purpose or rationale, the benefits, the risks, and the availability of alternatives. With this information presented, the patient herself can make whatever treatment decision seems best to her. If she were in a coma of some duration, then we might ask the husband under the doctrine of substituted judgment. But here, we can wake her up and ask her directly.

You see a patient in clinic – she has just received news that her quantitative hCG is declining and she is likely having a spontaneous miscarriage. She starts crying, “The Lord is punishing me! The Lord is punishing me!” What is the most appropriate response?

- A) “I can see that you're very upset – would you like to talk to our priest?”
- B) “This is a difficult situation, but there is no reason to think you can't carry a full term pregnancy”
- C) “Let's take a minute to pray together. That will help us decide where to go from here...”
- D) “This is a difficult situation. I'll allow you to gather your thoughts...”
- E) “This can just happen sometimes, and it's not your fault. I don't think the Lord has anything to do with this...”

Answer:

D) “This is a difficult situation. I'll allow you to gather your thoughts...”

“Silence is golden when you can't think of a good answer”
- Muhammad Ali

Give the patient a period of silence and allow her to process the news – then offer to educate her about what happened and what her options are going forward. Don't refer her to a priest without making sure the patient understands the situation and without at least offering your condolences first. Also, refrain from offering false hope by informing her that she can have a successful pregnancy – there is no way to make that promise to a patient. It is important to find out if the patient feels she is being punished for something, so if given the choice to ask the patient why she feels the Lord is punishing her, that would be appropriate as well. You should reassure her that the miscarriage is through no fault of her own, but it is best to first provide her comfort.

A 30 year old HIV-positive woman gives birth to a healthy baby boy. The woman has received no prenatal care. Tests are performed to assess the child's HIV status and return positive. She is clearly excited about the birth and appears to be very loving – she is constantly holding the baby and kissing him on the cheek every chance that she gets. When told of the infant's HIV results, the new mother appears oblivious, and says that she will just have to “be an even better mother to help get through this.” She asks about advice while breast feeding. The physician tells her that breast-feeding is not advisable, to which she replies, “I know that breast milk is best, and my baby deserves the best.” The physician's response should be which of the following?

- A) “I'm glad you are taking such good care of your baby. I'll schedule an appointment with the breastfeeding counselor.”
- B) “If you breastfeed your child, the courts can remove the child from your custody.”
- C) “If you really want what is best for your child, you will not breast-feed.”
- D) “Breastfeeding increases the risk of transmitting HIV to your child. You must not do it.”
- E) “It's great to see how happy you are – why don't we talk more about these things after you've had some rest.”
- F) “Let me explain. A positive test when the child is this young is not definitive. But if you breast-feed your child, you greatly increase the chances of your child contracting HIV.”
- G) “Yes, breastfeeding is best in most circumstances, but given your HIV status, I advise against it.”

Answer:

F) “Let me explain. A positive test when the child is this young is not definitive. But if you breast-feed your child, you greatly increase the chances of your child contracting HIV.”

Approximately 25% of untreated women with HIV will transmit the virus to their baby. All children of HIV-positive mothers will test positive at birth due to the mother's antibodies. Women who are HIV-positive should not breast-feed as that can increase the chances of congenital transmission by a significant degree. Courts do have legal precedent to remove children from the custody of mothers who insist on breast-feeding. Making sure the mother knows this is essential. However, the way that you tell the woman is also important. Direct commands are not the best option. Instead, explain the reasons for the recommendation in a way that make clear the risk to the child if the advice is not followed.

A 30 year old woman who was diagnosed with tuberculosis gives birth to a healthy baby boy. The woman has received no prenatal care. She asks about advice while breast feeding as she knows that is what is best for her baby. The physician tells her that breast-feeding is not advisable, to which she replies, "I know that breast milk is best, and my baby deserves the best." The physician's response should be which of the following?

- A) "I'm glad you are taking your new responsibilities so seriously. I'll schedule an appointment with the breastfeeding counselor."
- B) "If you insist on breastfeeding your child, the courts can remove the child from your custody."
- C) "Let me explain. If you breastfeed your child, you greatly increase the chances of your child contracting TB."

Answer:

- C) “Let me explain. If you breast-feed your child, you greatly increase the chances of your child contracting TB.”

There are some situations when you shouldn't breastfeed. This includes mothers who are HIV positive, undergoing chemo or radiation therapy, have untreated active tuberculosis, or are using illicit drugs.

Parents present to the emergency department with their 15 year old daughter. They are suspicious that their daughter has been sexually active with her 16 year old boyfriend, and request that you do a pelvic exam and urine pregnancy test on her. The patient, who is in high school and still lives with her parents, doesn't say much until you ask her about doing a pelvic exam, at which point she replies, "I'd rather you didn't..." What is the most appropriate course of action?

- A) Respect the minor's wishes, and defer all testing unless she consents
- B) Perform the urine pregnancy test but inform the parents that you cannot do a pelvic exam without consent
- C) Perform the pelvic exam but inform the parents that you cannot do the pregnancy test without consenting
- D) Inform the patient that since she is still a minor, she must comply with her parents' request

Answer:

A) Respect the minor's wishes, and defer all testing unless she consents

Typically, when a child refuses care (for instance a five year old who doesn't want sutures) – the parents can override and there isn't much of a dilemma. Problems arise when an adolescent refuses care. The refusal of care should be respected, under the mature minor doctrine, as the 15 year old is probably old enough to understand her actions. She certainly understands the nature and purpose of the examination. State law supports the minor when presenting for STDs and pregnancy. The child's refusal should be accepted under either the mature minor exception or may be covered under one of the specific treatment statutes with regard to pregnancy or STDs.

Mature minor doctrine:

The authority to consent or refuse treatment for a minor has traditionally remained with a parent or guardian. Over the years, courts have gradually recognized that children younger than eighteen years who show maturity and competence deserve a voice in determining their course of medical treatment. A minor who is found able to understand short and long-term consequences is considered to be "mature" and thus able to provide informed consent/refusal for medical treatment. The minor is authorized to make decisions regarding his or her medical treatment, so long as the following criteria are met: age > 14, capable of giving informed consent, treatment will benefit, treatment does not pose a great risk, and treatment is within established medical protocols. Although not every state has a mature minor doctrine, courts have recognized the need to look at certain case laws involving the ability of mature adolescents to make medical decisions.

You are seeing a patient in clinic who has been diagnosed with tuberculosis. He is an undocumented illegal immigrant. He is afraid of being deported if the Department of Health learns of his immigration status. What should you tell him?

- A) "You have nothing to worry about - the Department of Health does not ask for your immigration status."
- B) "As long as you remain compliant with treatment, there is no mandatory reporting to the government."
- C) "I will fully treat you and make sure you are taken care of before the government looks into sending you back."
- D) "It's important that you take the medications, and I'm sorry but legally I have to report this to the government."

Answer:

A) “You have nothing to worry about - the Department of Health does not ask for your immigration status.”

As a physician, you have an ethical duty to provide medical care to patients. Neither physicians nor the Department of Health report people's immigration status to the government. The Department of Health does not even ask about immigration status. There is no mandatory reporting to the government either before, during, or after the treatment of tuberculosis, regardless of compliance. A noncompliant patient might be incarcerated against his will to take TB medications, but they don't specifically face deportation for health reasons, and there is nothing in this question to indicate the patient will be noncompliant.

A 23 year old is in a serious motor vehicle accident and is pronounced brain dead. It is unclear whether he was registered as an organ donor or not, as his driver's license is not available and you must make a time-sensitive decision on whether or not he is a viable donor. His wife is unclear what he would have wanted, but she herself is a donor so she consents. Before anything can be done, his parents arrive and inform the doctors that they've never heard him discuss the issue of organ donation before and they would prefer that he not be made a donor. The wife has stepped out and is unavailable so you cannot get both parties together to discuss. What is the most appropriate course?

- A) Take the wife's consent and notify the organ donation network
- B) Take the parents' declination and do not notify the organ donation network
- C) Call the ethics committee

Answer:

A) Take the wife's consent and notify the organ donation network

The Uniform Anatomical Gift Act (UAGA) governs organ donation for the purpose of transplantation. The UAGA has created a very specific hierarchy of who can give consent for donation. Don't worry about memorizing this list, just understand that it exists and can easily be referenced. Keep in mind that if a person signs up to be a donor on the online registry, that constitutes irrevocable first person consent which only that person can rescind. This means that the person has made the decision to be a donor, and the family cannot overturn that decision.

The order of people who may decide to donate is:

- The donor him or herself
- Spouse
- Adult Children
- Parents
- Adult Siblings
- Adult Grandchildren

A 17 year old girl presents to your clinic. She is 12 weeks pregnant and would like to have an abortion. She does not want you to notify the father of her baby. What should be your response?

- A) “You are too far along, so I can't, in good faith, encourage you to get an abortion”
- B) “I can refer you to a specialist, but will need consent from your husband as well”
- C) “I can refer you to a specialist, and if you want me to maintain confidentiality from your husband, I will”

Answer:

C) "I can refer you to a specialist, and if you want me to maintain confidentiality from your husband, I will"

The legality of this issue can vary from state to state. Most states typically require one of two types of parental involvement – consent or notification, or both. 35 states require some type of parental involvement in a minor's decision to have an abortion – 22 states require one or both parents to consent to the procedure. The Supreme Court has ruled that parental involvement laws (and all other abortion regulation) can legally make it more difficult for a female to acquire an abortion. But there is a threshold beyond which the increased difficulties become unconstitutional. Requiring spousal involvement before a woman can acquire an abortion has been interpreted as unconstitutional while parental involvement has been interpreted as constitutional.

Planned Parenthood of Southeastern Pennsylvania v Casey (1992): Spousal notification laws place an "undue burden" on a woman's ability to get an abortion, whereas parental involvement laws do not.

A 15 year old male presents accompanied by his mother for his high school physical. She sits in the waiting room while you do the exam. Before you finish, the patient confesses that he is not aroused by females and thinks that he may be homosexual. He requests that you not tell his mother. When you finish the exam, his mother re-enters the room, and asks, "So, did you find anything? Everything a-okay??" How should you respond?

- A) Inform the mother of what he has told you, and encourage discussion between the two
- B) Tell her that everything is fine, but notify her later as she is the legal guardian and has a right to know. By not telling her in front of him, you maintain a strong relationship with him.
- C) Inform the mother that everything was fine and maintain confidentiality with the teenager
- D) Encourage the adolescent to tell his mother, but if he does not, then bring it up on your own

Answer:

- C) Inform the mother that everything was fine and maintain confidentiality with the teenager

The physician should not tell his parents about his patient's homosexual thoughts. He should encourage the patient to discuss his feelings with his mother, but in all areas dealing with sexual behavior, the minor has a legally protected right to confidentiality.

The truth is that many adolescents are not comfortable talking to their parents about controversial topics such as sex, drugs, peer pressure, etc. Studies show that adolescents are less likely to seek healthcare for sensitive issues if they believe that their parents will be informed. Many adolescents are unaware of their right to confidentiality, therefore physicians should discuss this with both the patient and their parents at their first visit so that everyone is aware of it. Limitations with regards to confidentiality should be explained. Parents and patients need to understand that if the adolescent poses a threat to self or others, confidentiality may be broken.

You are working in the emergency department when a patient comes in with a complicated laceration to his hand. You notify the orthopedic surgeon who comes down to see the patient. After examining and diagnosing him with a tendon laceration, he discovers that the patient is HIV positive. The physician asks that you refer him to another consultant, as he does not want to risk infecting himself by caring for this patient. Is he within his legal rights to refuse the patient?

- A) Yes, so long as it is not a life threatening situation physicians may refuse to see whomever they wish
- B) Yes, while it is unethical to refuse a patient he is within his legal means
- C) No, since the physician has formed a patient-physician relationship, he cannot abandon them unless it is outside his scope of practice
- D) No, he is violating the principle of malfeasance

Answer:

C) No, since the physician has formed a patient-physician relationship, he cannot abandon them unless it is outside his scope of practice

This question represents another gray area in medical ethics. Generally speaking, you should care for all patients that you find appropriate for your level of expertise. Refusing patients on any basis – be it racial, religious, sexual orientation – will open you up to scrutiny from the judicial system, ethics committee, and your colleagues. Moreover, once a doctor-patient relationship has been established, the physician cannot refuse to treat unless something falls outside of his scope of practice. To do so could be considered abandonment – if a physician wishes to terminate a relationship with a patient (for noncompliance for instance) the patient should always be notified well ahead of time so they are able to establish care with another provider. Ending such a relationship should be done both in person and through the use of a notarized letter so that there is no question about it.

There is a legal precedent for such cases – dentists have at times refused HIV patients on the basis that they don't have proper equipment/precautions to deal with such patients – however universal precautions are grounded in the fact that it will protect health care workers. Keep in mind that even if you don't violate the law, a lawsuit could be brought under the Americans With Disabilities Act (ADA). The ADA is a federal law that makes it illegal to discriminate against people because of their disability. The ADA prohibits a place of public accommodation (like a doctor's or chiropractor's office) from refusing to treat a person solely based on their disability. At the same time, a physician must voluntarily enter a relationship with a patient, and cannot be forced to accept new ones. This applies in cases where a physician feels his clinic is overbooked or is wanting to decrease his workload so he declines new patients – so long as it is done without prejudice it is wholly acceptable.

You are a primary care provider at a busy urban center. One of your patients happens to be the trauma surgeon at the hospital. He has been seeing you for routine care and recently had an HIV test done after having a high-risk sexual encounter without using protection. His HIV test is positive. Who are you legally obligated to inform?

- A) His insurance company**
- B) His patients, who might be at risk if he should cut himself during surgery**
- C) No one**
- D) His supervisor (chair of the department)**
- E) The hospital human resources department**

Answer:

C) No one

Patients with HIV have a right to privacy as long as they are not putting others at risk. You have no obligation to inform his insurance or his employer. You are not obligated to inform his patients as universal precautions are meant to prevail in order to prevent transmission.

In many European countries, including the UK and Scotland, physicians were long banned from practicing surgery or dentistry if they were known to be HIV positive. Such laws are in a constant state of flux but were still in place as recently as 2011.

A 12 year old boy is brought to the physician by his parents for a routine exam. You ask the parents to wait outside – when he is alone with you, the patient admits that he occasionally smokes cigarettes with his friends. When you initiate a discussion about smoking cessation, the patient says, “Smoking ain't a problem for me, doc...” Which of the following responses is most appropriate?

- A) “At what point will smoking become a problem for you?”
- B) “Did you know that smoking has many long-term health consequences?”
- C) “Do you want me to tell your parents about your smoking?”
- D) “You seem like a nice kid-- why did you start smoking?”
- E) “Why don't you just quit now before it gets to be a bigger problem?”
- F) “Don't you want to be able to run and play sports without getting short of breath?”
- G) “Let me show you some pictures of what happens to the lungs of people who smoke.”

Answer:

A) “At what point will smoking become a problem for you?”

“Teenagers have no sense of mortality – yours or theirs”
- Bono

You need to first understand what perceptions the child has about smoking. Once you have a grasp of what the child thinks about his habit, you can offer proper counsel. Discussing long-term consequences is not appropriate with adolescents, as they don't tend to think about long-term repercussions. Trying to instill fear into a patient (of any age) is never the right way to start off, and puts you at risk of losing your patient's trust. If you threaten to inform his parents now, he may not readily confess a different problem to you down the road. Inquiring about reasons for why the child started smoking is important, but is not the first step.

A 35 year old man comes to your office with a form to be filled as part of his pre-employment evaluation. He needs the form to give him a clear bill of health so that he can qualify for health insurance. The form also asks for an FAP gene test. This is in order for the company to determine which of its employees will need long-term healthcare. What should your response be?

- A) Perform the test and indicate the results on the form
- B) Perform the test but do not share the results with the employer
- C) Do not perform the test
- D) Ask the patient if he wants the test done and the results reported
- E) Include the test only if the patient has family members with the disease and is therefore at higher risk

Answer:

D) Ask the patient if he wants the test done and the results reported

Patients have a right to consent to any and all tests that are ordered, as long as they retain capacity to make their own decisions. Furthermore, patients have a right to confidentiality, and in most cases you must maintain this. The rare exception is where maintaining confidentiality either inadvertently puts others at risk or potentially may put the patient at risk. You may perform certain tests and report them to the employer with the consent of the patient.

Case:

You refer a 14 year old boy to a surgeon for repair of an inguinal hernia – it is an elective operation that will be scheduled for two months from today. The father provides consent for the surgery but the mother refuses on the grounds that she does not like the idea of her son having surgery. What should you do?

- A) Do the surgery as long as one parent consents
- B) Do not do the surgery since both parents need to sign consent
- C) Do the surgery if the child consents
- D) Obtain a court order to perform the surgery
- E) Do not perform the surgery unless strangulation occurs and it becomes medically necessary

Answer:

A) Do the surgery as long as one parent consents

You only need to obtain consent from one parent in order to make decisions regarding care of a child. Had this been a life threatening case, you would not need consent from anyone before treating. Encouraging discussion between the two parents would be the best place to start, but this was not given as an option. Ideally the child would also consent to the surgery, and his rights may be upheld under the mature minor doctrine, but abiding by his wishes (C) is not the best answer.

You are seeing a 70 year old male patient with progressive glaucoma. His vision is severely impaired and is worse compared to his last visit which was six months ago. On multiple visits in the past you have advised him to stop driving but he has not. You suspect that he now has difficulty even reading the traffic signs. What is your responsibility toward this patient?

- A) Maintain confidentiality but continue to encourage him to seek alternative modes of transportation
- B) Notify the patient's family of the risk he is putting himself at by continuing to drive
- C) Inform the patient that legally you must report him to the DMV
- D) Rescind his driver's license

Answer:

C) Inform the patient that legally you must report him to the DMV

In cases where patients' visual acuity is so severely impaired that you suspect he is a danger to himself and to others, you must encourage the patient to find alternative transportation. You do not have the right to remove or suspend driving privileges. You do, however, have a duty to report a visually impaired driver to the DMV so that the DMV can make its own determination of whether the patient's license should be removed.

Laws can vary from state to state – physicians should be aware of their professional responsibilities for the states in which they practice. A report to the DMV may be a service to the patient as well as to the public. While restricting driving privileges is almost certainly an inconvenience, the risk of injury or death to both the patient and third parties due to a medical impairment is too great a risk to ignore. Physicians should consider the options in their jurisdictions and keep the best interests of the patient and the public in mind.

A 30 year old male patient wishes to raise some money to pay off his college loans. He has a nephew who lives in Canada who is in need of a kidney and willing to pay \$20,000 for it. He comes to you for medical clearance. What should you tell him?

- A) It is okay to accept money if the recipient truly needs the organ
- B) It is never okay to accept money for the sale of your organs
- C) It is okay to be reimbursed for the cost of travel and lodging, but not to make a profit off the sale of organs
- D) It is okay so long as removing the organ you wish to sell does not put your life at risk
- E) It is okay to donate your organs, but it must go to whoever is next on the list to receive an organ – you cannot choose who receives it

Answer:

C) It is okay to be reimbursed for the cost of travel and lodging, but not to make a profit off the sale of organs

If one were allowed to receive cash for organs, it would create an unfair system whereby the wealthiest would receive transplants before the sickest. You can donate to whomever you like – for instance, if your brother is in need of part of your liver, you can choose to specifically donate that part of yours to him rather than have him wait until his name is at the top of the list for you to donate.

The wait list for each organ is formed differently, but the sickest people who are still strong enough to undergo surgery should receive the organs first. The only exception to this is with kidneys – since patients may survive on dialysis, the disbursement of kidney donations is on a first-come first-serve basis with the person who has waited the longest being on top of the list.

Steve Jobs:

Jobs needed a liver transplant – he couldn't legally pay for an organ, nor could he pay to cut the queue. He signed up at multiple transplant centers throughout the country to improve his chances. Health insurance policies often cover only one medical evaluation to get on one transplant center list. In 2006, the median number of days from joining the liver waiting list to transplant was 306 nationally. In Tennessee, where Jobs ended up having the surgery, it was 46 days.

Mickey Mantle:

Mantle suffered from liver damage mostly as a result of a long history of alcohol consumption and eventual metastatic cancer. The average wait in the Dallas area for a liver transplant where Mantle was hospitalized was 130 days. An organ was found for him the night after he was listed, leading many to suspect he was given preferential treatment. He died two months after receiving the transplant.

A 70 year old diabetic woman presents to the emergency room with an infection on her foot. X-rays and blood work confirm suspicion for osteomyelitis and the orthopedic surgeon is consulted. He recommends IV antibiotics and amputation of the foot. Informed of this, the patient refuses and says, "I know I'm going to die eventually, and I don't want to be footless when I do." Despite the IV antibiotics her respiratory status starts to decline and she is intubated for airway support. Her family arrives and asks that the physician amputate her foot in order to help stop the spread of infection and potentially save her life. What is the most appropriate course of action?

- A) Amputate her foot as she can no longer make decisions and her family is acting in her best interests
- B) Treat her with antibiotics and supportive care, but do not amputate her foot
- C) Consult ethics committee
- D) Find out if she has an advance directive or living will – and if that does not forbid amputation, only then should you proceed with surgery

Answer:

B) Treat her with antibiotics and supportive care, but do not amputate her foot

The patient made her wishes clear to you while she was able to, and there is no reason not to abide by them. Even if she had an advance directive indicating that she will consent to amputation in case of severe infection, the fact that she more recently refused such procedures will supercede the directive. A patient's right to make his or her own decisions – the principle of autonomy – is more important than substituted judgment or anyone else's opinion.

A 6 year old girl is brought to the emergency department by her mother because of "fever and a rash". The mother is a poor historian and does not offer up much additional information – she appears withdrawn and tearful. The child does not make very much eye contact with you and looks at the floor throughout her visit. She does not engage you in conversation. The most appropriate next step is:

- A) Admit the child to the hospital for evaluation and protection
- B) Ask if there is anyone else in the house that is sick
- C) Ask the mother and child separately what is concerning them
- D) Obtain a psychiatry consult
- E) Arrange for social services to visit the family at home

Answer:

C) Ask the mother and child separately what is concerning them

There appear to be other issues which need to be explored. Given the behavior of the mother and child, two things (of many) that need to be considered are domestic violence and child abuse. Each person may be afraid of openly talking about the problem in front of the other so it is best to talk to each individually. This will allow you to obtain additional information. Admitting the child to the hospital still leaves the mother vulnerable to domestic violence if that is the issue. Asking about sick contacts, while an important part of her medical history, is not the most appropriate next question. While a psychiatry consult may be necessary in the future, it is not immediately needed. If you suspect abuse, you should not send the child home with the parents under any circumstances.

A 40 year old woman comes to the office asking you to drug test her 15 year old son. She states that her son is normally a good student and is very interactive, but for the last three months he has become increasingly withdrawn. He spends more time in his room, his grades have dropped, and he does not spend as much time with his friends as he used to. She has confronted her son multiple times about his behavior, but he avoids talking about anything and denies using any alcohol or drugs. The mother appears genuinely concerned about her son and turns to you for help in figuring out what is wrong. How should you respond?

- A) "Bring your son in to see me and we'll start by ordering a drug and alcohol test to rule those out, with his permission"
- B) "I can't legally test your son for any drugs without him consenting to it first"
- C) "It's possible that your son may be suffering from depression and I think you should bring him in for me to talk to him"
- D) "It sounds like your son may be suffering from depression – I'd like to refer you to a psychiatrist who can talk to both of you together...."
- E) "The best place to start would be to setup an appointment with his principal at school to make sure there isn't something happening there that is negatively affecting him"
- F) "This is normal teenage behavior, but why don't you bring him in to see me just to be sure"

Answer:

C) "It's possible that your son may be suffering from depression and I think you should bring him in for me to talk to him"

The mother is right to be concerned over her son's change in behavior. While drugs and alcohol can cause behavioral changes, so can mental illness such as depression. As part of your workup for depression, you will check an alcohol and drug test, but failing to tell the mother that you are concerned about depression is impotent. If you only mention 'alcohol and drugs' this reaffirms her suspicions and she might not bring her son back to see you, thinking that she just needs to make him quit the substances he's not even using!

It is important to evaluate the patient on your own, without immediately referring to someone else. The mother should also set up an appointment with the school principal to address her concerns and find out if there is something else that she doesn't know about (perhaps the cause of his depression – whether or not he is picked on, if he has friends, reasons why his schoolwork may be suffering, etc) – but the physician should first speak with her son.

This question also brings up the issue of parents who request drug screens for their children. In such cases, it's important to educate the parents that a urine drug screen has both false positives and false negatives and cannot be reliably used to determine the presence or absence of illicit substances in all cases. So called 'designer drugs' are formulated in such a way that they are not detected on standard drug screens.

The minor should be questioned alone, ideally with the clinician sharing information about the parent's concerns. Minors often consent to drug testing, and when they do, the physician should first develop a plan for disclosure of test results to both parents and adolescent before ordering the test. For minors who refuse testing, it is rarely, if ever, appropriate to test. In cases of emergency where an adolescent is altered or unstable and you need to know about drugs that could be playing a role, it is prudent to perform a urine drug screen, just as in an adult patient.

A 35 year old male presents to your clinic for the first time. He has a history of HIV, and despite not taking any medications he has a high CD4 count and low viral load (in other words it is very well controlled). He is married and confesses to you that he does not always use protection when having intercourse with his wife, and that she is unaware of his HIV status. Since his CD4 count is so good and he does not require treatment, he sometimes forgets that he even has it. What is the next step?

- A) Encourage discussion between the husband and wife and strongly suggest he inform his wife of his HIV status
- B) Inform the wife yourself of your patient's HIV status
- C) Maintain confidentiality
- D) Have the Department of Health notify his wife
- E) If you can convince him to practice safe, protected sex – then there is no need for notification

Answer:

A) Encourage discussion between the husband and wife and strongly suggest he inform his wife of his HIV status

Always encourage discussion between husband and wife first. HIV is a mandatory reportable disease, and you will need to notify the Department of Health – but the first step is educating the patient on why it is important to inform his wife, and giving him the opportunity to do so. If he had been given this advice before, and continued to refuse to discuss the issue with his wife, you would need to take it to the next level and notify the Department of Health. However the question clearly states that this is the first time the patient is seeing you so it is reasonable to give him a chance first.

A 55 year old patient presents to the emergency department complaining of chest pain. After obtaining an EKG, you inform the patient that he is having an ST elevation myocardial infarction (STEMI) and will likely need further intervention. You inform the patient that the risks of having an angiogram include development of a hematoma or coronary rupture – and the benefit would be that it would stop the chest pain and could provide definitive treatment of the infarction. He opts to forgo the angiogram because of a fear of adverse effects and you proceed with medical management. He eventually goes into cardiac arrest and dies, and his family brings a lawsuit against you. What will be the most likely outcome?

- A) You are not liable because he refused the treatment that was offered
- B) You are not liable because you followed the 'informed consent' standard and documented your conversation with the patient
- C) You are liable because patients cannot refuse life saving therapy, and the patient had no real chance of survival without the angiogram
- D) You are liable because you did not inform the patient of the risks of forgoing angioplasty

Answer:

D) You are liable because you did not inform the patient of the risks of forgoing angioplasty

The patient was never fully informed. Nowhere in your conversation did you inform the patient of the risks of not having the recommended treatment (angiogram). Since he was not aware that he would likely die without this, he was not fully informed which makes you liable.

For consent to be legitimate, it has to contain the following elements:

- The nature of the procedure
- The most significant risks of the procedure
- The benefits of the procedure
- Any possible alternatives to the procedure, including the risks of not having anything done (when that is a legitimate option)

Bubb v Brusky, 2009:

Patient was diagnosed with a TIA after his symptoms resolved and a CT scan of his head did not show any acute findings. The physician discussed the case with a neurologist who scheduled close outpatient followup, and he was discharged home. The next day the patient suffered a stroke – filed a lawsuit claiming that the physician did not inform him of the alternative to outpatient management of TIA, including admission, MRI, and possible carotid ultrasound. The patient alleged that he would have stayed had he known that was an option, which would have prevented the stroke. Wisconsin Supreme Court found in his favor: “any physician who treats a patient to inform the patient about the availability of all alternate, viable medical modes of treatment, including diagnosis, as well as the benefits and risks of such treatment.”

A patient presents to your clinic and informs you that he is an active heroin drug user. He has a history of insulin-dependent diabetes but in the five years you have known him he has never once been compliant with his medications. Regarding his IV drug abuse, he admits that he sometimes has difficulty getting clean needles, and asks if you can either prescribe him some syringes or refer him to a free clinic where he can get some. When you start to lecture him on why he shouldn't use drugs, he says, "okay, okay – then let's just pretend it's for something else. Give it to me for the diabetes." What should you do?

- A) Prescribe him sterile syringes
- B) Inform him that since it is against the law to use drugs, you cannot encourage him by providing him with easier access
- C) Advise that he go to the health department where they may be able to help

Answer:

A) Prescribe him sterile syringes

Offering the patient sterile syringes will reduce his risk of acquiring HIV or hepatitis. The patient should be referred to appropriate health facilities, whether you agree with this from a moral standpoint or not. It follows the basic principle of medicine: 'first, do no harm'. Refusing to give the patient syringes will not dissuade him from using drugs – you aren't doing anything to try to rehabilitate your patient. In this case, your options are to give him syringes and encourage him to be safe in whatever habits he has, or to turn him away and have him fend for himself.

A patient presents to your clinic for follow up on test results. He receives a new diagnosis of Familial Hypercholesterolemia. The patient has recently become divorced and understands that the disease follows an autosomal dominant pattern of inheritance which places his children at high risk. He refuses to give consent to inform his ex-wife who now has custody of their children. He threatens legal action if you reveal any part of his medical history to his ex-wife. What should you do?

- A) Respect the patient's right to confidentiality
- B) Have the patient talk to a cardiologist so that he can better understand the seriousness of his condition and its inheritance patterns before making a decision
- C) Ask the health department to inform the ex-wife
- D) Seek a court order to inform the ex-wife
- E) Inform the ex-wife of the risk to the children
- F) Inform the ex-wife's primary care physician so that he can test her for the disease

Answer:

E) Inform the ex-wife of the risk to the children

The patient's right to confidentiality ends when it comes into conflict with the safety of other people. The children have a right to know whether their lives will be adversely affected by the disease. A standard part of the agreement for most divorces is the stipulation that each parent must inform the other parent of health care issues for their children.

The health department does not do notification of genetic diseases. It notifies partners and the population at risk of transmissible diseases such as tuberculosis, HIV, STDs, and food and water-borne illnesses. Simply notifying the wife's physician is inappropriate as the children also need to be tested; in the event that the wife decides to change to a different physician or is unaware of any need to see her physician she might never talk to the one person you communicated with.

The American Medical Association's (AMA) code of ethics emphasizes that physicians should inform all patients, who are considering undergoing genetic testing, of the circumstances under which they would be expected to notify biological relatives of information related to risk of disease.

Safer v Estate of Pack:

The court held that a physician's duty to warn might not be fulfilled by telling only the patient, rather – all of those who were potentially at risk of developing the condition. The plaintiff in Safer suffered from multiple colonic polyps and subsequent cancer – a result of a hereditary condition that her father had been treated for when she was a minor. Lawsuit alleged that the disease was hereditary and that the physician had breached duty to inform those who were potentially at risk of developing the condition by only informing the patient. The court employed the concept of a “genetic family” - the idea that genetic information is not just personal medical information but is also familial.

You are seeing a 60 year old male in your clinic following an inpatient admission. He has been hospitalized multiple times in the past few months with complicated urinary tract infections, and has finally completed a prolonged course of home antibiotics and seems to be doing much better. At the outpatient clinic appointment, his wife hands you two tickets to a football game and states 'my husband and I want to thank you for everything...' She informs you that her husband works at the stadium and receives two free tickets to each home game. He adds that the games are boring and he usually sells the tickets, but wanted you to enjoy them. What is the most appropriate response?

- A) "Thanks, but it's against my policy to accept expensive gifts"
- B) "Thanks – I can't go but my Dad loves football and I'm sure he'll enjoy these...."
- C) "Thanks – I'll have fun at the game; but you should know that I give you the same level of care I give all of my patients"
- D) "Thanks - I don't normally accept gifts, but since they were free to you I'll accept them"
- E) "Thanks – how much do you normally sell them for?"

Answer:

A) “Thanks, but it's against my policy to accept expensive gifts”

The legality of accepting gifts can vary from office to office. Some have a strict policy of never accepting gifts from patients, others set a limit on how much value one can accept from patients. As a general rule, it is permissible to accept small tokens of appreciation – such as a box of cookies for instance. The value for a pair of football tickets should be considered too high – as evidenced by the fact that the patient normally sells them for a profit. The American Medical Association (AMA) recommends considering how accepting or rejecting a gift will have a positive or negative impact on the patient-physician relationship. For instance, in some cultures it might be considered customary to express appreciation by gifting a small token. Declining the patient the right to express such gratitude could be insulting and harm your relationship with him. As a general rule, never accept cash gifts. Gifts should not influence any decisions you make with regards to care of your patients.

A 16 year old female presents to your clinic accompanied by her mother. She has lost weight and her mother admits that she is always complaining of abdominal pain and refusing to eat. She is dizzy upon standing, weak, and orthostatic. After completing your assessment, you suspect that she has anorexia nervosa. You explain to her that her pain is likely from delayed gastric emptying and impaired intestinal motility and that she will need hospitalization for treatment. The patient gets very upset, shouting, "You can't do this to me! It's my body and I can do whatever I want!" How should you respond?

- A) Encourage the patient to stay for hospitalization, explaining the benefits of treatment
- B) Allow the patient to leave, but have her sign an AMA (against medical advice) form
- C) Since her mother is her legal guardian, ask the mother to make a decision
- D) Consult ethics committee
- E) Speak privately with the mother to ascertain additional information

Answer:

- A) Encourage the patient to stay for hospitalization, explaining the benefits of treatment

The patient has put her health in jeopardy – she is dizzy, weak, and orthostatic – allowing her to continue down this road untreated is irresponsible. However, in yet another gray area of medical ethics, it is difficult to justify keeping the patient against her will. Since her life is not in danger, or at least is in danger almost as much as someone who smokes, you can't keep her against her will. Encouraging her to be hospitalized is the right thing to do, and making sure she is fully informed of all of the benefits of being admitted is appropriate. Speaking privately with either party might provide additional history and can be done at some point, but is not the next most appropriate course of action. Paternalism is when the physician makes a decision regarding what is best for a patient on his own - this is in contrast to autonomy which grants patients the power to make their own decisions. Patients with eating disorders meet all the conditions required to give informed consent and hence should have capacity.

You are a surgeon caring for a 40 year old male who is admitted for abdominal pain. The patient's CT scan shows inflammation surrounding his appendix, and you are planning on taking him to the operating room for an appendectomy at the next available opening in the schedule. After informing the patient of your plan, as you exit the patient's room you encounter his wife who has just arrived at the hospital. She quickly asks what has happened to her husband and what is planned. What is the most appropriate course of action?

- A) Inform the patient's wife of his condition as she is his legal next of kin
- B) Inform the patient's wife that you will need consent from her husband before discussing
- C) Inform the patient's wife that her husband has appendicitis, but also tell her that she must discuss any additional details with her husband
- D) Inform the patient's wife that her husband has abdominal pain, but do not give any additional information

Answer:

B) Inform the patient's wife that you will need consent from her husband before discussing

A patient must give consent prior to disclosing his or her confidential medical information. Sometimes patients wouldn't want other family members to know the details of their care or even the fact that they are in the hospital. Without knowing what sort of relationship the patient and his spouse have, you can't assume that he would want her informed.

HIPAA (Health Insurance Portability and Accountability Act): the first national legislation to assure every patient has protected health information. Hospitals and healthcare workers must inform patients in writing of how their health data will be used, establish systems to track disclosure, and allow patients to review and obtain copies of their own health information. HIPAA made it illegal to disclose a patient's health information to any person who is not a member of the healthcare team.

You are caring for an inpatient male infant who was born premature. He is underdeveloped and now, after several months, is starting to have problems with urination. He has a history of recurrent kidney stones. His mother insists that you use natural treatments and suggests Pulsatilla flower, which may be of benefit. She finds out you have only ordered IV fluids and antibiotics and is upset that you are not encouraging the nurses to use the homeopathic remedies. She questions your credentials and requests that you transfer her son to another hospital. Which of the following is most appropriate?

- A) Explain that the Pulsatilla flower can cause vomiting, diarrhea, and convulsions – and is therefore not a recommended treatment
- B) Inform the mother that you will order the use of the natural treatment, but in reality continue treatment without it
- C) Ensure the mother that you are competent and well-trained, and that her child is getting the best care possible
- D) Agree on a plan whereby you treat her son's symptoms your way for now, then use the mother's plan once the child is a little more developed and has lower risk for side effects

Answer:

- D) Agree on a plan whereby you treat her son's symptoms your way for now, then use the mother's plan once the child is a little more developed and has lower risk for side effects.

In order to be an effective physician, you must be accepting of others' beliefs. This will facilitate forming a connection with your patients. Without knowing what kind of background your patients come from, you should not attempt to discredit or devalue their suggestions. So long as there is no harm done to the patient, it is advisable to work together with the family to agree on a plan of care. Never order a treatment without informing the family what you are doing, and never mislead them by giving false information. Emphasizing your credentials is not satisfactory – the mother is not focused on a perceived lack of education in her physician – she is concerned that you are not using her form of therapy.

A 75 year old woman is admitted to the hospital following a massive stroke. She was intubated in the emergency department upon arrival for airway protection. She does not possess a living will or advance directive. She has two children – her son, who is the elder, is the first to arrive and informs the physician that he would like a DNR order placed. Next, the woman's daughter arrives and claims that her brother isn't interested in helping their mother. He had been the one to encourage the other family members to put her into a nursing home. She wants everything done to extend her mother's life. The mother was widowed and hadn't specified which of her children was to make decisions on her behalf. You have both siblings meet but they cannot agree. What is the most appropriate course of action?

- A) Maintain the DNR order since the first-born child has priority to make decisions in such cases
- B) Remove the DNR order in accordance with the daughter's wishes since she is more interested in helping her mother
- C) Using the substituted judgment standard, make a decision as to whether or not to maintain a DNR order
- D) Consult the ethics committee

Answer:

D) Consult the ethics committee

In cases where a spouse is not involved, the responsibility for end of life decisions belongs to a patient's children. Unless one is specifically designated as a healthcare proxy by the patient prior to her condition, all of the children carry equal weight in decision making. Using substituted judgment is the proper thing to do, however there is nothing in the stem of the question to indicate what the patient would have wanted. For instance, if they can recall ever watching a television show with their mother, where one of the characters was on a ventilator and she commented how she would never want one herself – that anecdote alone would hold significant weight. Therefore the children should be asked to recollect any such examples if they can – remember, substituted judgment is not asking what you or anyone else would want done, rather what the patient would have wanted if they could make the decision. If an agreement cannot be reached, the ethics committee should be asked to weigh in.

An orthopedic surgeon comes to your office as a patient. He has a history of seizures as a child, and has had about one per year since medical school. He is on staff at your hospital, and admits that during a surgery recently he had a complex partial seizure that lasted thirty seconds. He had a brief alteration in consciousness that went unnoticed and this was the first time it had happened at work. He wants you to start him on antiepileptics as he has never taken them before, and plans to have his wife drive him to work from now on. What should you do?

- A) Since you work at the same hospital, you cannot maintain confidentiality, so help set him up to see a physician at an outside facility
- B) Inform his supervisor (hospital administration)
- C) Prescribe him anti-epileptics but maintain confidentiality
- D) Prescribe him anti-epileptics, and if he has any breakthrough seizure, then break confidentiality
- E) Since his seizures are so infrequent, inform him that he will not require medication to manage his condition

Answer:

B) Inform his supervisor (hospital administration)

Hospitals have well documented policies regarding the 'impaired physician' - one who is unable to fulfill professional responsibilities because of psychiatric illness, alcoholism, or drug dependency. The same principle applies here – you have a physician who cannot safely perform his job without placing his patients and others at undue harm. Imagine if, as an orthopedic surgeon, he is operating on a hip when he has a seizure and has transient loss of consciousness! The protection of patients in the operating room trumps doctor-patient confidentiality. Unless you can guarantee that proper treatment would safely allow the surgeon to practice (for instance if he was a psychiatrist, having a temporary loss of consciousness would not place his patients in any danger) his condition should be disclosed to the hospital. In this case, you should first encourage the surgeon to report himself. If he refuses, you are obligated to go to the hospital administration against his wishes.

A 30 year old woman comes to your clinic with a two week history of acne and blisters on her forehead and cheeks. She appears very upset and cries, “I look like a strawberry!” What is the most appropriate initial response?

- A) “How many hours a week do you spend in the sun and do you use sunscreen?”
- B) “I understand that you're upset, but let me assure you, this is something that we can treat.”
- C) “I understand that you're upset, after all - a rash on the face can be embarrassing.”
- D) “I can prescribe an ointment to help get rid of the rash – but it will take some time and patience.”
- E) “Do you have any family history of skin cancer?”
- F) “What medications do you take?”

Answer:

- C) "I understand that you're upset, after all - a rash on the face can be embarrassing."

Whenever a patient is upset, the first step in alleviating their concerns or putting their mind at ease is to acknowledge their problem. Sometimes, simply repeating what they've just told you is the best course of action. Asking questions to try to move through a differential diagnosis is important, but not the best place to start. Ensuring the patient that this can be treated is imprudent and not advised, since you cannot guarantee that this can be treated.

A 30 year old male is diagnosed with tuberculosis. He is an undocumented illegal immigrant who has never seen a physician before as he can not afford to miss time at work. He asks the physician not to report the diagnosis to anyone because he is afraid he will be deported. Which of the following is most appropriate?

- A) Do not report the case and have the patient wear a mask at all times
- B) Do not report the case as long as the patient agrees to isolate himself while being treated
- C) Do not report the case, but contact the patient's family members and work site directly to arrange for evaluation
- D) Report the case to the health department only
- E) Report the case to the health department to ensure that family members and coworkers are identified and evaluated
- F) Report the case to the immigration and naturalization service (INS)

Answer:

E) Report the case to the health department to ensure that family members and coworkers are identified and evaluated

Failing to report the case unnecessarily puts others at risk. Tuberculosis is a mandatory reportable illness, and all close contacts need to be notified so that they can take proper precautions. There is no need to notify the immigration and naturalization service, as this responsibility does not fall on the health department.

A 60 year old patient is admitted for constipation and GI bleeding. He is found to have a mass on CT scan of his abdomen/pelvis. The next step in workup is to perform a colonoscopy and biopsy. While obtaining consent, the patient states, "Please don't tell me what it is – if it's cancer, I don't wanna know..." What is the most appropriate reply?

- A) "If you don't want to know the results, or act on them, then there is no reason to do the procedure"
- B) "Do you have any family members that you would like to have make health care decisions for you?"
- C) "You are competent to make health care decisions so I am legally obligated to inform you of your results"
- D) "I will need to get a psychiatrist to evaluate if you are competent to make decisions or not"
- E) "I'd like to discuss the findings with your primary physician, if that's okay"
- F) "Are you having any thoughts of wanting to hurt yourself?"

Answer:

B) “Do you have any family members that you would like to have make health care decisions for you?”

Just as competent patients have the right to refuse medical care, they also have the right to refuse knowledge of their diagnosis if they so choose. The patient should be aware of the risks of refusing this knowledge and attempts should be made to identify a surrogate decision maker for the patient so that medical care can proceed. In general, your first response should be an open ended question meant to encourage the patient to discuss the reasoning behind his decision. Since this is not an option, the next step is to ask who can make decisions on his behalf for his treatment.

You admit a 76 year old male with pneumonia. His past medical history is significant for coronary artery disease, diabetes, and hypertension. He admits to smoking a pack of cigarettes daily. You start treatment with antibiotics and inhalers. Which is the most important topic to discuss with the patient at this time?

- A) Education about quitting smoking to lessen the chance of dying from heart or lung disease
- B) Ask about an advance directive and the patient's wishes for end of life care
- C) Education about the need to exercise to improve hearth health
- D) Ask about a will so that if something happens during the hospitalization, his affairs are in order
- E) Ensure the patient has refills so that he is compliant with his medications when discharged

Answer:

B) Ask about an advance directive and the patient's wishes for end of life care

All of the other issues are nonacute and therefore nonessential to discuss during the admission process. All patients, when admitted, should be asked about advanced planning and the presence of an advance directive. There is no way to predict what will happen during a patient's hospitalization, and now is the time to clarify what he or she would want done should the need to make decisions arise.

A 45 year old male presents to the clinic for a routine physical examination. His past medical history is unremarkable. Although he has no complaints, he appears more anxious than normal. His elderly father was recently diagnosed with prostate cancer. He sometimes lays awake at night scared and requests blood work to ensure that he does not have it as well. The patient states, "I can pay out of pocket for whatever test is needed..." Digital rectal examination is unremarkable. Understanding that a PSA level is not normally done until after age 50, what is the most appropriate course of action?

- A) Refer the patient to a urologist
- B) Start checking PSA levels when the patient reaches the age of 50
- C) Reassure the patient that he is at low risk for prostate cancer because he is asymptomatic and has normal DRE
- D) Check the PSA level today
- E) Have the patient return if he develops any symptoms

Answer:

D) Check the PSA level today

The patient is clearly anxious and it is starting to affect his activities of daily life. In order to relieve his anxiety, it is prudent to order the test today. Immediate referral without a clear indication is never the right answer.

A previously healthy 45 year old male had a sudden onset generalized headache. He lost consciousness and collapsed shortly thereafter. On arrival to the emergency department he is unresponsive – his CT scan shows a large intracranial bleed and he is presumed to have a subarachnoid hemorrhage from a ruptured berry aneurysm. He quickly goes into cardiac arrest and is pronounced dead. His wife and two children arrive at the hospital – when offered an autopsy on her husband's body, the wife replies, “I don't know what to do....” What is the most appropriate response?

- A) Attempt to contact his parents for consent
- B) Have the organ donation network discuss options with the wife
- C) Perform genetic testing on a postmortem blood sample
- D) Contact the medical examiner to perform a mandated autopsy

Answer:

D) Contact the medical examiner to perform a mandated autopsy

The law requires that a medical examiner investigate any death occurring suddenly and unexpectedly or from an unexplained cause. This includes deaths of individuals who are found dead without obvious cause, and medically unexpected deaths which occur during the course of medical treatment or during the course of therapeutic or diagnostic medical procedures. Medical examiner autopsies assist in determining cause and manner of death. Permission of the next-of-kin is not required.

Under normal circumstances, a wife's consent takes precedence over that of the parents. Contacting the organ donation network is inappropriate as you need to identify a cause of death – for instance, since infection (mycotic aneurysm) is in the differential, this would need to be ruled out prior to any consideration for organ donation.

A 50 year old woman presents to your clinic and admits that her spouse regularly abuses her both physically and emotionally. She is well known to you and has a long-standing history of chronic alcohol abuse. Which of the following is the most appropriate initial response by the physician?

- A) "Abuse will only get worse with time – for your safety you should leave the relationship"
- B) "There may be a correlation between your history of alcoholism and the abuse"
- C) "Were you ever abused as a child?"
- D) "Do you feel safe at home?"
- E) "Do you know why he abuses you?"

Answer:

D) “Do you feel safe at home?”

While all of the above are good questions to ask, the most important thing to do is to ensure the patient's immediate safety. If she does not have a safe place to stay you can offer her social services or shelters and make an immediate impact on her situation. The remainder of the options, while appropriate, are meant to help her deal with the problem on a longer-term basis.

A 30 year old Hindu woman was recently diagnosed with acute leukemia. She insists on seeing a homeopathic traditional healer rather than undergoing chemotherapy. Which of the following responses is most appropriate?

- A) "I recommend that you see an oncologist – if you disagree then you should seek treatment elsewhere"
- B) "This is a big decision – I'd like to have you see a psychiatrist to make sure you understand everything before making a choice"
- C) "I can recommend a traditional homeopathic healer that I've worked with in the past..."
- D) "In addition to seeing a traditional healer, I would like you to consider chemotherapy"
- E) "Chemotherapy is the only realistic chance you have of treating your leukemia"

Answer:

D) “In addition to seeing a traditional healer, I would like you to consider chemotherapy”

You should never refuse to see a patient or try to refer them to another physician, unless the case is outside of your expertise. There is no indication that the patient needs to see a psychiatrist, rather, this is a case of differing cultural backgrounds. Recommending a traditional healer and ignoring the suggested chemotherapeutic regimen sends the wrong message to the patient and goes against what you really feel will help the patient. Likewise, ignoring her suggestion will likely lead to noncompliance, mistrust, and a feeling of degradation. The best choice is to incorporate both options so long as the homeopathic treatment does not interfere with the chemotherapy regimen.

An unconscious 40 year old male is brought in with a gunshot wound to the abdomen. His blood pressure is 70/p and he will require a blood transfusion to survive. He is wearing a shirt that says “Don't Taze me Bro – I'm a Jehovah's Witness”. As you are deciding what to do, his wife arrives and states that it is against their religion to give blood transfusions, and despite the fact that he might die without it, he would not have wanted it. Should you give the transfusion?

A) Yes

B) No

Answer:

A) Yes

Jehovah's Witnesses don't accept blood transfusions based on the Bible's command to "abstain from blood" - therefore, they do not accept blood in ANY form.

Such patients typically carry identifying cards or something else to verify their belief. It's considered general practice to have a card indicating advance directive to refuse blood. Since there is no way to confirm with the patient, there are countless possibilities that should dissuade you from just obeying the spouse and withholding lifesaving treatment – they may be going through divorce, she may want to see her spouse dead, she may be the one who shot him, etc.

A 25 year old patient comes to see you in the clinic. She has had several positive home pregnancy tests which are confirmed by a urine dipstick in your office. She admits to being intoxicated and having unprotected intercourse with her husband two months ago. She is still in school and not ready to start having a family. She asks if you can recommend a doctor for her to see about an abortion. She admits that her husband drove her today, is in the waiting room, and is ecstatic at the prospect of being a father. He is absolutely opposed to her having an abortion. What is your legal obligation?

- A) None – you don't have to discuss anything with the husband.
- B) You must inform both the husband and the wife of the risks of an abortion, and allow them to make a decision together. Ultimately, the decision belongs to the wife.
- C) You must inform both the husband and the wife, and if she still wants to proceed he may obtain a court order to prevent the abortion.
- D) Find out more about why she wants the abortion, and try to convince her otherwise.

Answer:

A) None – you don't have to discuss anything with the husband.

Courts have consistently decided that a woman's right to an abortion can't be vetoed by a husband, partner, or ex-boyfriend, and also that a woman doesn't have to notify the father that she intends to have an abortion.

Planned Parenthood v. Danforth (1976) and Planned Parenthood v. Casey (1992):

Supreme Court ruled that requiring women to notify fathers of abortion placed too great of a burden on women. The Supreme Court ruled: "it cannot be claimed that the father's interest in the fetus' welfare is equal to the mother's protected liberty..."

You are a physician attending a fundraiser when one of the wealthy donors goes into cardiac arrest. You start CPR and he regains a pulse before paramedics arrive. His wife is grateful and writes you a check for \$500 to cover the cost of your seat at the fundraiser. A few months later you receive a notice in the mail indicating that you are being sued. You are shocked to discover that the man suffered a pneumothorax from your aggressive compressions. Are you liable?

- A) No, because he survived and suffered no serious long-term complication
- B) No, because you performed to the best of your ability and if you had not intervened, he could have died
- C) Yes, because you accepted compensation
- D) Yes, because the complication was a direct result of your actions

Answer:

C) Yes, because you accepted compensation

In most states, accepting compensation or sending a bill complicates the issue of whether you had a pre-existing duty to provide care to the individual, and therefore might revoke your Good Samaritan immunity.

Who is the Good Samaritan?

A man was traveling from Jerusalem to Jericho when robbers attacked him and left him for dead by the side of the road. A priest happened to be traveling the same road, but when he saw the man he passed by on the other side. Likewise a Levite also came, and when he saw the man he walked by on the other side. But a Samaritan traveler who came upon him cared for his wounds and took the man to an inn. The next day he took out two silver coins and gave them to the inn-keeper saying, "Take care of him. If you spend more than what I have given you, I shall repay you on my way back."

Noble v Sartori (1990):

Two brothers were working together when one developed chest pain and felt like he was having a heart attack. They went to the nearest hospital – someone 'dressed like a physician' (white coat, stethoscope) passed by as they were in the waiting room and the patient's brother asked him for help. The physician, who was not on emergency room duty at the time, replied, "Get in line and sign in." The physician then walked away. Frustrated, they went to another hospital where he died the next day of a myocardial infarction. A malpractice lawsuit was filed against the physician, who responded by saying that no physician-patient relationship had been formed, therefore he was not liable. The court ruled in favor of the physician: 'although we find [the defendant physician's] callous disregard for the patient to be morally reprehensible, we can find no legal duty to treat a non-patient.'

Laws vary from state to state but Minnesota in particular has a statute obligating a person who encounters a medical emergency to provide assistance to the best of their ability. A physician (or anybody) can be held liable if they make no effort to assist in an emergency, if it can later be shown that they had a reasonable opportunity to do so.

A woman is driving when she observes another car swerve to avoid a deer and go into a ditch. The car rolls over once but lands with the proper side up. The woman stops to help and although there are no immediate signs of danger she notices the driver has a wrist deformity and is therefore unable to get out of the car. The woman climbs into the passenger side and pulls her out. She then calls 911 and paramedics spinally immobilize the patient and take her to the hospital. Later, it is determined that the woman had a cervical spine fracture and is now paralyzed. She sues the woman who originally stopped to help her. Is the woman protected under the Good Samaritan law?

A) Yes

B) No

Answer:

B) No

There was no danger of remaining in the car - Good Samaritan provisions are not universal in application. In the absence of imminent peril, the actions of a rescuer may be perceived by the courts to be reckless and therefore not deserving of legal protection. In this case the intentions were good, but the court can rule that Good Samaritan laws do not apply because the victim was not in imminent peril and hold the actions of the rescuer to be unnecessary and reckless.

Let's look at three of the same cases with different outcomes:

- Displacing a cervical spine fracture in the ER = malpractice
 - Displacing a cervical spine fracture in a roadside accident where there was no harm in leaving the patient in the car = likely malpractice
- Displacing a cervical spine fracture in a roadside accident where the patient was in imminent danger = protected

A 65 year old man suffered a stroke three months ago. He has persistent right sided weakness as a result. He is at increased risk for which of the following?

- A) Major depressive disorder
- B) Obsessive-compulsive disorder
- C) Post traumatic stress disorder
- D) Antisocial personality disorder
- E) Social phobia
- F) Agoraphobia

Answer:

A) Major depressive disorder

Major depressive disorder is considered the most frequent and important neuropsychiatric consequence of stroke. As high as 33% of stroke survivors experience major depression, and this tends to peak 3-6 months after stroke. As many as 50% of cases resolve within the first year. The higher the severity of stroke (ie loss of activities of daily living), the higher the incidence of depression.

One of your colleagues is out of town so you are covering for his patients. A gentleman presents with low back pain. In his chart, you see that he has received x-rays, an MRI, and has been treated with several different pain medications unsuccessfully. He has asked for referral to a neurosurgeon despite negative imaging, to see if anything else can be done – but his doctor has held off on doing so. The patient appreciates you giving him a refill of naproxen, but starts to talk about how he doesn't trust his regular doctor and doesn't think he is a qualified physician. What should be your response?

- A) Notify your colleague after the patient has left
- B) Advise the patient to notify his doctor of his concerns
- C) Advise the patient to establish care with a new doctor
- D) Inform the patient how to properly file a complaint so that his concerns can be addressed

Answer:

B) Advise the patient to notify his doctor of his concerns

The best first step is to encourage the patient to bring up his concerns with his regular doctor. If still unsatisfied, he can file a complaint or choose to see a different doctor – but you should always encourage open communication as a first resort.

75 year old male presents to the ER in respiratory distress and unable to answer questions. According to his chart he has a history of metastatic lung cancer for which he recently completed chemotherapy. He does not have an advance directive or living will. His wife and son are also present, and request that you not intubate him as his cancer is so advanced and he has 'lived a full life and does not need to suffer any more'. What should you do?

- A) Intubate the patient anyway
- B) Respect the family's wishes and treat him as much as possible short of placing him on a ventilator
- C) Ask the family to put their interests aside and think about what the patient would have wanted
- D) Contact the patient's primary care physician and see if he has ever communicated his end-of-life wishes to him
- E) Obtain a stat ethics committee consultation

Answer:

- C) Ask the family to put their interests aside and think about what the patient would have wanted

Respect for patient autonomy is the dominant principle in medical ethics. In the hierarchy of decision making, the advance directive is the first thing to take into consideration. Substituted judgment is the second line approach, but it's important to determine if the person making the decisions is putting the patient's interests first. In this case, the patient has completed chemotherapy which demonstrates an interest in being treated – had the patient suffered from end stage cancer and been refusing all forms of therapy, that should make you reconsider the decision to intubate. Moreover, the family clearly states 'we don't think he would want to suffer any more' – there is never any indication that the patient himself had expressed these wishes. The third approach to use is that of 'best interest' – which has several limitations and in this case is difficult to determine. In cases of emergency, it's always best to treat first and ask questions later, because it's much harder to question now and treat later.

A surrogate decision maker should attempt to establish (with as much accuracy as possible) what decision the patient would have made if the patient were able to do so. This can be based on previous statements expressed or the surrogate's knowledge of the patient's values, beliefs, personality, and prior lifestyle. Surrogacy seeks to preserve the patient's right of self-determination by placing the patient's own preferences at the center of the decision.

A 10 year old girl is brought to the emergency room for a foot infection – she will require IV antibiotics and debridement or is at risk for amputation. The mother is at the bedside and declines antibiotics on the grounds that she has several allergies to different medications, as does her daughter, and she is concerned that her daughter will develop an allergic reaction. She will consent to the debridement however. Which of the following is the most appropriate course of action?

- A) Perform the debridement and withhold antibiotics unless the infection worsens
- B) Perform the debridement and administer antibiotics contrary to the mother's wishes
- C) Ask the patient if she will consent for antibiotics
- D) Refuse to perform the debridement unless you can also administer antibiotics, as the one treatment is ineffective without the other
- E) Attempt to obtain consent from the father

Answer:

B) Perform the debridement and administer antibiotics contrary to the mother's wishes

In all situations involving life or limb-threatening cases in minors, physicians may treat without consent. Performing only the debridement and waiting to see if the infection worsens is incorrect and medically negligent. Remember, first do no harm. Although it is a good idea to keep the patient informed as well, obtaining consent from the patient is unnecessary. Consent from one parent is adequate and if this were not a life or limb threatening case, this could be correct – however there is no indication that the father is at the bedside or readily available so it is imprudent to attempt to contact him – you should treat irrespective of consent.

An 80 year old male presents in respiratory distress from end stage COPD. You decide that he will require intubation. While preparing your equipment, his wife and son arrive and inform you that he does not have an advance directive or living will, but that 'he would not have wanted to be hooked up to machines'. The patient is unable to appreciate the situation and therefore cannot offer much perspective one way or the other. What is the most appropriate course of action?

- A) Inform the family that you must intubate the patient for now, and you can then discuss whether or not he would have wanted this and how to proceed
- B) Do not intubate the patient, but admit him to the hospital and make his status do-not-resuscitate (DNR)
- C) Do not intubate the patient, and inform the family that he will likely die without this. Admit him to the hospital if he survives that long

Answer:

C) Do not intubate the patient, and inform the family that he will likely die without this. Admit him to the hospital if he survives that long

Without an advance directive or living will, spouse and then children should use the principle of substituted judgment to determine how to proceed. In this case, it is reasonable to see how the patient would not have wanted to be intubated so the proper thing to do is to keep him comfortable. There is no indication that he requires a DNR status however. Nowhere in your conversation with the family did you discuss administering antiarrhythmic drugs or cardiopulmonary resuscitation (CPR) – therefore, should he go into cardiac arrest it would still be proper to use such measures. Ideally you would clarify that with the patient's family immediately after deciding not to intubate as that is the likely next step.

A patient presents to the ER following an allergic reaction to shrimp. He is in distress and will likely need intubation. Although he is currently protecting his airway and able to speak, his tongue is swollen and seems to be even moreso compared to when you first saw him fifteen minutes ago. Under normal circumstances you would intubate the patient – however he has a history of being intubated last year following angioedema secondary to ACE inhibitor use. He had a prolonged course and ended up receiving a tracheostomy – after which he was so put off by the whole thing that he had an advance directive made outlining that under no circumstance would he want to be intubated again. When the nurses are out of the room gathering medications, and you are alone with the patient, he is able to tell you 'Doc, I decided I want the tube again. I've changed my mind about that whole advance directive thing.' What should you do?

- A) Respect the advance directive and treat him with everything short of intubation
- B) Put off intubating as long as possible – but if he continues to decompensate and you have no other option, do it
- C) Reject the advance directive and intubate
- D) Flip a coin

Answer:

C) Reject the advance directive and intubate

Advance directives only apply if a patient is unable to make personal medical decisions. As long as a patient remains able to participate in medical decisions, an advance directive can be revoked, and informed decisions by competent patients always supersede any written directive.

60 yo male is in the preoperative area. He has a history of advanced COPD, diabetes, and hypertension, and has been admitted for a mass that was found by colonoscopy. He is about to undergo a partial colectomy despite being considered a high risk surgical candidate. After discussing his diagnosis and the planned treatment, the patient admits that he is afraid something may go wrong during the surgery and asks that you pray for him. He has very different religious beliefs than you, and you are not sure how to respond. What is the most appropriate thing to say?

- A) "I will be happy to call a chaplain for you"
- B) "I am not religious but I will pray for you if you think it will be helpful"
- C) "I will be happy to, but just so that you know, my beliefs are different from yours"
- D) "I understand your beliefs are very important, and I will keep you in my thoughts"

Answer:

D) “I understand your beliefs are very important, and I will keep you in my thoughts”

Physicians should at least agree in a generic sense to keep the patient in their thoughts and prayers – this will appease him, make him more comfortable, and improve your patient-physician relationship. Deferring to a chaplain may defeat the purpose – if the patient is more comfortable with you and will feel stronger knowing he is in your thoughts, don't abandon him. For the purpose of boards, a good habit to get in the practice of is to ask yourself, “What would Jesus do?” 9 times out of 10 this will lead you to the correct answer.

You are seeing a 15 year old male in the clinic. He has a history of seizures and has been prescribed phenytoin for management. You check a blood level which comes back low, and you suspect that he has been noncompliant. After informing the patient that his phenytoin level is low, which of the following questions is most appropriate?

- A) "How often do you miss taking a dose?"
- B) "Taking phenytoin every day can be difficult - what's it like for you?"
- C) "In my experience, a low level is usually an indication that patients have trouble taking their medicine as prescribed – do you have this problem?"
- D) "If, for some reason, you're having trouble taking your medicine, we can have one of your parents manage it for you to make sure that you take it every day"
- E) "You seem to have missed a few doses – do you understand the importance of taking it?"
- F) "Phenytoin can have a lot of side effects, but if you don't take it you can have another seizure, which would be much worse"

Answer:

B) "Taking phenytoin every day can be difficult – what's it like for you?"

What would Jesus do?

There are a few simple rules to follow for most ethics questions. The first is to never assume a patient is noncompliant. Whether it's an insulin-dependent diabetic whose sugar is high or it's an epileptic teenager whose phenytoin level is low – there are a myriad of factors which could lead you to falsely accusing a patient of noncompliance. Chronic alcoholism, use of certain antibiotics, and high doses of salicylate (aspirin) are only a few of those which can lower phenytoin levels. Giving up on a patient for noncompliance before allowing him to explain himself is inappropriate.

Taking control away from the patient and giving it to his or her parents will only antagonize your relationship. Teenagers often battle for control and cherish the opportunity to make decisions for themselves. Taking that away only increases the chances of noncompliance and also has an underlying tone of mistrust. Finally, scaring a patient into taking medication is never the right answer.

A 25 year old female, accompanied by police officers, is brought to the emergency room. She was the restrained driver in a motor vehicle accident earlier, but has no medical complaints. She smells of alcohol and admits to drinking tonight. She is alert and oriented. The police officer requests that you draw a blood alcohol level but the patient does not want this done. What is the most appropriate action?

- A) Order the nurse to draw the blood and send it to the lab
- B) Order the nurse to draw the blood but inform the officer that you cannot send it to the lab without the patient's consent
- C) Refuse to draw the blood without consent from the patient
- D) Ask the police officer if he has a search warrant. If he does not, refuse to draw the blood. If he does, then proceed with drawing blood
- E) Explain to the officer that blood alcohol levels are notoriously unreliable, and cannot be used alone to convict someone

Answer:

D) Ask the police officer if he has a search warrant. If he does not, refuse to draw the blood. If he does, then proceed with drawing blood

Laws vary state to state, and this is an issue that is always changing. There is no correct answer 100% of the time, but ethically speaking, you should never perform a test on an oriented patient without their consent. The only exception would be a test whose findings might lead to a life-saving measure (ie child who refuses CT scan after significant head injury). This particular blood test is not being used to help or treat a patient in any way.

The Fourth Amendment to the Constitution states: 'The right of the people to be secure in their persons, houses, papers, and effects, against unreasonable searches and seizures, shall not be violated...'

People v Farris, 2012

Defendant was involved in an accident where she sustained only minor injuries. She refused to consent to a blood draw, so the officer ordered hospital staff to pin her arms and legs down while a draw was performed. Court found that Illinois law, although it does not grant a person the right to refuse, also does not grant police the right to use force to draw blood.

22 year old male who plays quarterback for the college football team presents to your clinic for a sore throat. He has a high fever, tonsillar exudate, and cervical lymphadenopathy. After drawing some blood, you diagnose him with infectious mononucleosis. Mono may cause an enlarged spleen, which is at risk to rupture if you are hit in the abdomen. After informing him that he will need to avoid all contact sports for the next four weeks, he exclaims, "But the state championship is next week! I gotta play doc!" You inform him that you don't think it's safe, and he replies with a wink, "well, since this is just between you and me, you know, with all that confidentiality stuff, we'll see...." What is the most appropriate response?

- A) Document in his chart that you discussed the risks of playing so that you are legally covered
- B) Call the student's parents and inform them of his visit and your concerns
- C) Call the student's coach and inform him or her of your concerns
- D) Re-examine the patient in one week and determine if he is able to play
- E) Inform the patient that you used to play football and have a great three point shot – then offer to fill in for him

Answer:

C) Call the student's coach and inform him or her of your concerns

Most student athletes sign an authorization form that allows information from team doctors or athletic trainers to be shared with coaches. This is meant to protect the patient – athletes with mononucleosis must refrain from physical activity for a minimum of 21 days. At that point a repeat physical examination and possible abdominal ultrasound to assess for splenomegaly can be performed. However it would violate the standard of care to allow a patient to resume athletic activities only one week after being diagnosed.

A 40 year old woman presents to the emergency room with constipation. She does not speak English but has her 16 year old son with her. You take a history using her son and it's now time to examine her. What should the physician do?

- A) Have her son translate**
- B) Call a hospital translator**
- C) Ask one of the nurses, who happens to speak the same language, to translate**

Answer:

B) Call a hospital translator

Always use an official translator when possible – this is the only way to ensure that everything you are saying is being communicated to the patient. Family members, regardless of age, might leave out certain details or improperly translate some of the words you are saying, altering their meaning. Also, they will likely not understand the need for word-by-word translation and might cut corners on what they inform the non-English speaker. Using a nurse is also not correct because they may not be able to translate specific medical words which are not typically taught in classes. It is entirely possible to be fully fluent in a language yet not be able to inform a patient of their test results.

A six year old child accidentally witnesses his parents having sexual intercourse. The parents are unsure how to explain to the child what happened, and call you for advice. How should you respond?

- A) Parents should inform the child that they were just 'wrestling around' and 'play fighting'
- B) Parents should inform the child that they were just 'showing each other how much they love one another'
- C) Advise them to take the child to a psychiatrist
- D) Parents should ask the child what he thinks he saw
- E) Parents should avoid talking about it as it is inappropriate for someone his or her age to have to deal with such a sensitive topic

Answer:

D) Parents should ask the child what he thinks he saw

The appropriate response depends upon the child's development and understanding. The most important thing is for the child to understand that what he saw was not unfriendly or hostile – in other words he needs to know that his parents were not trying to hurt one another. It is also important to get some idea of how the child viewed the incident as this insight can prove valuable to understanding his or her level of comprehension.

In general, if children have been exposed to parental intercourse they need an age-adequate explanation and reassurance about any misconceptions they may have with regards to what they saw.

You are eating lunch at a restaurant when a nearby customer slumps over. You rush over to help, and find the person unresponsive. Additionally you note that he has no pulse. You are a family medicine physician and don't typically encounter such emergencies. Nonetheless, you perform CPR to the best of your ability, alternating compressions and rescue breathing at a ratio of 15:2. The paramedics arrive but the patient does not survive. Later, while you are telling your story of heroism, they correct you and note that the new recommendations are to perform this resuscitation at a ratio of 30:2. Are you liable or are you protected under the Good Samaritan law?

- A) Liable because you did not follow the standard of care
- B) Protected because you acted to the best of your professional ability
- C) Protected because if you had done nothing, the patient would have had no chance for survival

Answer:

B) Protected because you acted to the best of your professional ability

Aside from the obvious defense that performing compressions at the proper rate would probably not have changed any outcome, you have not had any reason to take the new basic life support course – and are performing life-saving maneuvers to the best of your ability. Therefore you could expect to be protected by the Good Samaritan law in your state. Good Samaritan laws protect health care providers unless it can be proven that there was a gross departure from the normal standard of care or willful wrongdoing on the provider's part.

Ordinary negligence is when an individual fails to act as a reasonable healthcare provider would under certain circumstances (standard of care). Gross negligence is when an individual not only fails to follow the standard of care, but his or her actions are malicious (failing to do CPR because you recognize a patient as a known drug dealer).

Gordin v William Beaumont Hospital:

A patient was involved in a motor vehicle accident and taken to the emergency department, where it was determined that the patient would need to go to the operating room. The on-call surgeon was unavailable so another surgeon, who was not officially on call, was contacted and agreed to come to the hospital. He evaluated the patient and the decision was made to operate. The patient eventually went into cardiac arrest and died. The physician used the Good Samaritan defense and won the case – hence the law may even protect physicians who respond to emergencies when not 'on duty'.

You are seeing a five year old boy in the emergency department after he tripped on the stairs. He has bruises over different parts of his body that appear to be of different ages. He is accompanied by his father, who seems very caring and concerned about the child's injuries. You suspect child abuse based on the pattern of findings and decide to confront the father about your concerns. He is taken aback at the accusation and adamantly denies ever hurting his child. What is the most appropriate course of action?

- A) Obtain x-rays and if your workup does not reveal any findings, discharge the patient home with his father
- B) Notify child protective services
- C) Call the police
- D) Speak to the child alone about whether or not someone is abusing him
- E) Admit the child to the hospital to remove him from a potentially dangerous environment

Answer:

B) Notify child protective services

Child abuse is a mandatory reportable offense. Since there is no way to know everything about a home situation, it is always better to err on the side of caution and the law protects you from any potential false allegations that you make. The father may be unaware that his spouse is abusing the child, or an elder sibling is abusing the child – so it is inappropriate to discharge him home. Speaking to the child alone will be important and can be done with the help of a social worker as well, but regardless of what the child tells you, it is still imperative that you involve child protective services. You do not have the authority to remove a child from the care of his parents.

Maples v Siddiqui:

Family sued a physician alleging that the physician negligently diagnosed their child's malnutrition as being caused by inadequate parenting skills. After the child was placed in temporary foster care, it was discovered that the child was suffering from malabsorption syndrome and liver cirrhosis. The child was then returned to his parents. The court found in favor of the physician, arguing that good-faith reporting of suspected child abuse or neglect should protect a physician from liability. The court concluded that the legislature granted immunity with the understanding that a physician might be negligent, and that to permit liability would discourage those who suspect child abuse from making a report. The physician had immunity even when there was a failure to diagnose.

A first year intern is on call overnight for his internal medicine rotation. It's a particularly slow night, so after doing a few admissions and playing a game of table tennis with the other interns on call, he finally finds some time to take a nap. He is awakened by his pager at 3am, and it's a nurse on one of the floors calling to inform him that one of the patient's he is on coverage for is complaining of chest pain. He asks the nurse to give the patient a dose of morphine and to call him back if the pain is unrelieved. The next morning an EKG is done by one of the nurses and the patient is found to have an ST segment elevation myocardial infarction (STEMI). Who is most likely to be sued?

- A) The intern who was on call
- B) The nurse who was caring for the patient
- C) The attending who is ultimately caring for the patient
- D) None of the above

Answer:

A) The intern who was on call

You may have heard the phrase “time is heart” or “time is brain” in cases of either a heart attack or stroke: both diagnoses are time-sensitive and it is of utmost importance to not delay treatment. In such a case, someone is definitely liable to a malpractice claim. The nurse who was caring for the patient did the right thing by alerting the on call intern. The patient should have been interviewed and decisions regarding EKG, x-ray, lab tests, aspirin, etc should have been made. For years, courts have treated medical residents, even first-year residents, as true physicians when it comes to the professional standard of care in medical malpractice cases. A lack of experience does not lower the standard of care. By the end of 2006, the National Practitioner Data Bank had cataloged more than 1800 residents as having had at least one malpractice claim against them.

Lilly v Brink:

In this case the resident diagnosed indigestion and released the patient, who died later that day from a cardiac event. The court determined that the resident used his own discretion in diagnosing, treating, and releasing the patient. The court viewed this performance as equal to that of any fully licensed physician, so the resident should also be treated as one.

Centman v Cobb:

Court found that first-year residents are practitioners of medicine, required to exercise the same standard of skill as a physician with an unlimited license to practice medicine. The court stated that as a health care practitioner, a first-year resident who assumes treatment and care for patients “impliedly contracts that she has the reasonable and ordinary qualifications of her profession and that she will exercise reasonable skill, diligence, and care in treating the patient”.

You are a cardiologist invited to attend a dinner at a nice steak restaurant. You discover that the evening is being sponsored by a pharmaceutical representative, and the purpose of the dinner is to listen to a presentation on new treatments for hyperlipidemia. You also learn that they will be giving out \$300 tablets as an incentive. Which of the following is most appropriate?

- A) Decline the invitation to the dinner altogether
- B) Go to the restaurant but decline dinner and the tablet gift
- C) Go to the dinner but decline the tablet gift
- D) Go to the dinner and accept the tablet gift, but do not allow them to influence which medications you prescribe

Answer:

C) Go to the dinner but decline the tablet gift

It is permissible to attend a lecture and receive dinner for being part of an audience at a presentation. Accepting either cash or items which are worth significant amounts of cash is not ethically upheld.

A neurologist is asked to give a grand rounds presentation at an outside hospital. He is researching new treatments for epilepsy, and one of the anti-epileptic companies is sponsoring his presentation and providing him with \$2000 compensation. They do not ask for a copy of his slides or have any input into his presentation. What should he do?

- A) Decline invitation to give the lecture
- B) Give the lecture but decline the money
- C) Give the lecture, accept the money, but inform the audience of the compensation
- D) Give the lecture and accept the money
- E) Give the lecture and accept the money, to be used only towards the research

Answer:

C) Give the lecture, accept the money, but inform the audience of the compensation

It is permissible to accept compensation so long as you disclose any financial or business connections that you may have with the audience. This allows the audience to make a fully informed decision regarding how much credence to give your data. It is important that your presentation be free of bias and that you should maintain control of its content – if you can't ensure this, then you should probably reject the invitation.

A deaf patient presents to the emergency department with a sore throat. He is accompanied by his wife who can both hear and perform sign language (ie interpret). The patient does not appear to be in any distress and through the wife you are able to obtain a history. You order a rapid strep test which comes back positive and plan to administer antibiotics and discharge him. He is afebrile and vital signs are normal. Which of the following is most appropriate?

- A) Communicate your plan to the patient through his wife
- B) Communicate your plan to the patient by writing
- C) Wait for a sign language interpreter to come in and serve as your source of communication
- D) Speak very slowly and allow him to read your lips, explain your plan, and administer antibiotics
- E) Print out the discharge paperwork and give ample time for him to read through that until he sufficiently understands everything that you have done for him

Answer:

- C) Wait for a sign language interpreter to come in and serve as your source of communication

Whenever you encounter a patient who either speaks a different language or uses a different form of communication, as in this case, you must always use a properly licensed interpreter who has both experience and knowledge in working in the medical field. Without any way to verify what his wife has communicated to him, there is no way to obtain an adequate history. Also, you don't know what kind of relationship he has with his wife and you should never make assumptions. If a sign language interpreter was not available, communicating by writing – while slowing down the process – would be the preferred choice.

A 30 year old male is brought to the ER by ambulance. His girlfriend is present at the bedside and notes that when he awoke this morning he was confused. He is employed as a lawyer, has no medical problems, takes no medications, and does not drink or use any drugs. You determine that he will need a CT scan of his head. The patient has some difficulty following instruction however and will not lay still for the scan. What is the most appropriate course of action?

- A) Search for other causes of altered mental status. If you cannot find another etiology, admit to the hospital for neuro checks
- B) Physically restrain the patient to the bed and ensure that he cannot move his head so that you can obtain the scan
- C) Obtain consent from his girlfriend to restrain him and perform a lumbar puncture instead
- D) Restrain him and perform a lumbar puncture instead, regardless of whether the girlfriend provides consent
- E) Chemically sedate the patient and obtain the CT scan

Answer:

E) Chemically sedate the patient and obtain the CT scan

In cases where the patient is disoriented and does not have capacity to make decisions for his own care, you must proceed with your emergent workup as you normally would. The next step is to either obtain a CT scan of his head or just to do the lumbar puncture – either way consent from his girlfriend offers you no protection whatsoever. Physically restraining the patient so that he cannot even move his head, but leaving him awake, might be considered cruel, extremely time consuming, difficult to perform, and maybe even impossible. Since the patient may have an infection or intracranial bleed, this is a time-sensitive issue. Chemically sedating the patient and obtaining a CT scan is the next best step. Some might argue that administering sedatives to an already altered patient can muddy up the picture – however, if it's necessary to obtain a scan in an urgent manner, which in this case it is, knowing whether or not the patient is decompensating is less important. Worst case scenario, you can always intubate the patient to obtain whatever tests are necessary.

You are seeing a 50 year old mentally retarded patient in your clinic. He lives alone and has a friend who comes to check in on him every once in a while. He does some volunteer work at the church and in exchange he is given free meals. He has a mental age of 10 and can neither read nor write. You discuss the need to do a PSA level and digital rectal exam as part of his routine medical screening. When you explain the rectal exam to him, his eyes get big and he says, "I don't want it! I don't want it!" What should you do?

- A) Perform the PSA if he will consent, but do not do the digital rectal exam
- B) Perform both the PSA level and the digital rectal exam
- C) Attempt to obtain consent from family members
- D) Show him pictures and thoroughly explain both tests to him. If he still refuses then document his refusal

Answer:

D) Show him pictures and thoroughly explain both tests to him. If he still refuses then document his refusal

Informed consent involves discussion of the purpose of your test, the risks/benefits of both doing the test and of not doing the test, and alternatives. You must make sure your patient has a thorough understanding of the issue before accepting his refusal. Performing a rectal exam on a patient can be considered assault. It is not appropriate to physically restrain a patient in order to do so, while the patient still has capacity to make healthcare related decisions. While the patient clearly has significant cognitive impairment, there is nothing in the question to indicate he should be considered incompetent to make medical decisions.

A 45 year old woman that you recently diagnosed with breast cancer presents to your clinic. She is behind on payment of her medical bills so has not scheduled an appointment with you in several weeks. She requests a copy of all of her medical records so that she can take them to her attorney and determine if there is enough to bring a lawsuit against you for failing to make the diagnosis earlier. You attempt to calm her down but she is verbally abusive and is adamant on getting copies of all of your notes. What is the most appropriate course of action?

- A) Have her sign consent and provide her with copies of all imaging studies and lab results
- B) Have her sign consent and provide her with a copy of her entire medical chart including your notes
- C) Do not give her a copy of the chart but inform her that her attorney may contact you for it
- D) Do not give her a copy of the chart until past money owed has been repaid
- E) Do not give her a copy as it can only be used against you to strengthen a potential lawsuit

Answer:

B) Have her sign consent and provide her with a copy of her entire medical chart including your notes

A medical record can never be withheld from a patient – consider it the property of the patient and you must surrender a copy any time the patient signs consent. Regardless of what she intends to do with it (establish a new primary care provider, bring litigation against you, etc) – it is her right to have a copy of her entire medical chart. In fact, if another physician were to ask you for a copy of her records, she is the one who would have to provide consent – therefore it would be wise to consider the record the property of the patient.

You are an intern on your ICU rotation. You attend a birthday party and see your supervising resident drinking alcohol, while you remain sober knowing that you have to go to work the next day. The next day, you see the resident at work and seeing patients. She appears to be fine. However, at one point she is supervising nurses and running a code blue, only to have the patient die. What should you do?

- A) Inform your attending of what happened the night before and of your concerns
- B) Do not inform anyone
- C) Discuss your concerns with the resident herself
- D) Notify the residency director

Answer:

B) Do not inform anyone

There is no indication that the resident is an impaired physician. What she chooses to do with her time off is not of anyone's concern, so long as she is not putting anyone else in jeopardy. If she was intoxicated at work then you have a duty to both discuss the issue with her and to notify her superiors, however, an unsuccessful cardiac resuscitation does not imply impairment. If the resident came in with the smell of alcohol on her breath or appeared intoxicated, you would have a duty to inform the residency director (her superior, not the attending physician). Failing to do so would make you potentially liable for any harm that came to her patients. Even if you discussed your concerns with her and she acknowledged that she had a problem and would take steps to fix it, you would still need to notify the residency director.

A 65 year old woman comes to your clinic for exhaustion and fatigue. Her husband died three months ago in a motor vehicle accident while she was a passenger in the car. She admits she sometimes cries for no reason and sometimes hears his voice. What is the most appropriate response?

- A) "Do you have flashbacks to the accident or nightmares at night?"
- B) "I'm going to give you a medication that will help you feel better and will help you sleep at night."
- C) "Do you think about your husband often?"
- D) "I'd like to schedule you an appointment with my colleague who can help us understand why you're feeling this way."
- E) "I see a lot of patients for depression, so let me assure you that you are not alone."

Answer:

C) “Do you think about your husband often?”

This is a normal grief reaction, which can last for up to one year. Invite the patient to talk about her feelings.

The American Psychiatric Association (APA) considered changing the definition of depression in the new DSM-5, which would specifically characterize bereavement as a depressive disorder. In removing the 'bereavement exclusion', the DSM-5 would encourage clinicians to diagnose major depression in patients with normal bereavement after only two weeks of mild depressive symptoms - medicalizing normal grief and labeling healthy people with a psychiatric diagnosis.

In May 2012, the APA announced that although the bereavement exclusion will be eliminated from the definition of major depression, a footnote will be added indicating that sadness with some mild depressive symptoms in the face of loss should not necessarily be viewed as major depression.

A 30 year old male jumps off of a bridge in an attempt to end his life. He survives, but suffers a pelvic fracture in the process. He is brought in by paramedics and is hypotensive. While the trauma surgeon is informing him of the need to go to the OR and obtaining consent, the patient refuses to sign the forms – indicating that he understands he will die without the surgery but he does not wish to live any longer and therefore does not wish to undergo surgery. He is willing to stay in the hospital for pain medication however. What is the appropriate course of action?

- A) Take the patient to the operating room anyway
- B) Obtain a court order to take the patient to the operating room
- C) Find out if the patient has a wife or parents who might be able to provide consent
- D) Document your conversation with the patient, indicating that he understands and accepts the risks, and admit him to the hospital without going to the operating room

Answer:

A) Take the patient to the operating room anyway

Suicidal patients lose the right to make medical decisions for themselves – especially in the case of a life threatening emergency. They are presumed to lack decision making capacity. In this case, a young patient with a clear indication to undergo surgery should have no delay and should be taken right away. Contacting other family members and obtaining court orders unnecessarily delays the process and is not legally necessary.

A 28 year old Somali woman is brought to the clinic by her husband for cough and shortness of breath for 2 days. You (a male) enter the room. The patient appears distant and shy – her husband introduces himself and states that he brought her. He asks “what can you do to help my wife?” For every question that you ask, he answers. The patient herself avoids all eye contact. What is the most appropriate step?

- A) Directly ask the patient to describe her symptoms
- B) Ask the patient if she wants to have her husband present during the exam
- C) Ask the husband to wait outside so you can examine her in private
- D) Explain to the husband that the most complete history would come directly from the patient, and question her about her symptoms

Answer:

C) Ask the husband to wait outside so you can examine her in private

This exonerates the patient from having to have a potentially difficult conversation with her husband. If she would rather that he stay, she can object. It might help if you keep a female nurse with you, so that he doesn't suspect you are wanting to be in the room alone with his wife. This may just be a cultural difference, but you want to ensure there isn't something else that you are missing.

A 16 year old female presents to your office for vaginal discharge. The patient admits to recent unprotected sex and after performing a pelvic examination and wet mount, you suspect that she has gonorrhea. What is the most appropriate next step?

- A) Treat her infection and just have her follow up as needed
- B) Ask her to refer her sexual partner so that you can examine him as well. After examining both, decide what the optimal course of treatment would be
- C) Treat her infection and discuss the importance of testing for other STDs
- D) Notify her parents, but also provide treatment for her infection

Answer:

C) Treat her infection and discuss the importance of testing for other STDs

The next best step is to test the patient for other potential sexually transmitted diseases. According to the CDC, chlamydia is the most commonly reported STD in the United States while trichomoniasis is the most common STD in sexually active women. Treating the patient's infection is certainly of utmost importance, but it is inappropriate to merely treat her for gonorrhea and send her home without further testing. There is no indication that you need to inform her parents, as minors can receive treatment for sexually transmitted infections without parental consent. Advising the patient to have her partner tested is also advisable, but you would not withhold treatment on the basis that the patient must be present with her partner.

A 26 year old woman comes to you (male physician) for nontraumatic back pain. Her exam is unremarkable. You suggest anti-inflammatories and muscle relaxers. As the patient is leaving, she asks if you ever date your patients and if you would agree to see her socially. Which of the following is most appropriate?

- A) Do not pursue any social relationship with her, but tell her that you would like to continue being her physician
- B) Agree to see her socially as long as she understands that it is separate from the medical relationship
- C) Explain that no social relationship is possible as long as she is your patient
- D) Refer the patient to a colleague and begin to see her socially
- E) Refer the patient to a colleague but still decline to see her socially

Answer:

A) Do not pursue any social relationship with her, but tell her that you would like to continue being her physician

Agreeing to see a patient socially raises all kinds of moral/ethical issues. Telling her that you can not see her as long as you are her physician leaves the door open, and suggests that if she gets a new physician you will socially see her. Once a physician-patient relationship has been formed, that person should be regarded as the physician's patient for life. The patient may choose to see another physician, but the physician should never end this relationship.

Dupree v Giugliano:

In 2000, Kristin Dupree received treatment for depression and stress from James Giugliano, a licensed family physician. He prescribed antidepressants and referred her to a therapist for counseling. In 2001, they became involved in a sexual relationship, but after nine months they mutually agreed to end the affair. She confessed the adultery to her husband, who subsequently sued for divorce. In 2005 she filed a malpractice lawsuit claiming that the affair was wrong but that she was unable to control herself. An expert witness testified that her romantic feelings were the result of “eroticized transference,” a medical phenomenon in which the patient experiences near psychotic attraction to a treating physician, which the patient is powerless to resist. She was awarded more than \$500,000.

A 35 year old male presents to your clinic to follow up on some lab results. He was found to have iron deficiency anemia. He readily admits that he does not eat meat due to religious reasons, and has never felt comfortable swallowing pills so it is difficult to maintain adequate iron levels. What is the next most appropriate question?

- A) "If you don't start eating the right things now, you're only going to have more problems later in life."
- B) "I can write a note to your religious leader saying that it's medically necessary for you to be exempt from normal dietary restrictions."
- C) "Why does your religion prohibit eating meat?"
- D) "Would you like information on other things you can eat to keep your iron levels up?"
- E) "Whenever something is dangerous to your health, your religion will permit you to make an exception."
- F) "If you don't get your levels up, you might need blood transfusions and shots – it would be much easier to try swallowing the pills."

Answer:

D) “Would you like information on other things you can eat to keep your iron levels up?”

Regardless of your religious beliefs, it's important to always be accepting of others' views. There is no need to question their religion or try to convince them to do things that contradict their particular set of beliefs. Rather, try to work with them to find a solution that will allow them to maintain their values while not jeopardizing their health. Threatening the patient with what may happen to him later in life is inappropriate as it gives the impression that you are offering an ultimatum – contradict your religion or risk your health. It puts the patient in a difficult position and is not the best way to work through this.

Imagine your religion forbids you from doing something, and someone offers to write you a note excusing you. This isn't missing a day of work! That note is not going to get you anywhere. Asking a patient additional questions about his or her religion is not a bad idea and will strengthen your relationship and level of understanding – but is not the most appropriate question to work towards finding a solution to this particular problem.

A 70 year old male is found to have a pulmonary nodule on chest x-ray. Further workup (biopsy, CT scans) reveals metastatic cancer. As you are approaching the room to discuss the findings, the patient's son stops you outside and says "If it's cancer, please don't tell him." What is the most appropriate response?

- A) "I'm sorry, I'm required to tell my patients everything"
- B) "Talk to me about why you don't want me to tell him"
- C) "I need to tell him what we know – if he is okay with it you're more than welcome to come in while I discuss the findings"
- D) "Why don't we go into an empty room and I can tell you what we know so far"

Answer:

B) "Talk to me about why you don't want me to tell him"

Find out what concerns the son has if his father were to find out. Patients have a right to all information regarding their medical care, but you may learn something from the son that will change the way you present the results or deal with the patient's response. For instance, if you learned that the patient may become depressed if his results have come back a certain way, you would still be obligated to inform him but it would certainly make you more aware of the patient's emotional status.

A 65 year old male is hospitalized for weight loss and dysphagia. New diagnosis esophageal cancer is reached – after extensive workup it is determined his life expectancy will be about six months. As you enter the room to tell him the news, his family is sitting around his bed. What is the most appropriate response?

- A) “I have some bad news to tell you.”
- B) “The results of your tests are back. Your cancer is very advanced and the results suggest you have about six months left to live.”
- C) “I have the results of your tests. Would you like your family to stay or would you prefer they stepped out?”
- D) “I have the results of your tests. I'd like to discuss them with you privately.”

Answer:

D) “I have the results of your tests. I'd like to discuss them with you privately.”

By asking the family to leave, you're absolving the patient of that responsibility. You're preventing him from having to confront his family. If the patient does want his family to stay, he can always override you and invite them to stay, but at least this way he has that option. It's important to emphasize that family support can never be overestimated. With the patient's consent it's always best to deliver bad news in the presence of another close friend or family member.

A 16 year old girl comes to your clinic. She is healthy but is unaccompanied by an adult. She has never been pregnant and is on her period. She denies being sexually active but requests oral contraception. What is the most appropriate response?

- A) "I would be happy to discuss this with you. Please have your parents accompany you on the next appointment and we can take it from there"
- B) "I will prescribe the oral contraception to you"
- C) "I will prescribe the oral contraception to you, but must notify your parents if they should inquire about your visit"
- D) "Since you are not sexually active, there is no indication for contraceptives and therefore I can't prescribe them"

Answer:

B) “I will prescribe the oral contraception to you”

In cases involving contraception, STDs, prenatal care, or substance abuse – it is legal and appropriate to treat while respecting confidentiality even if the patient is under the age of 18. Encouraging her to have open discussion with her parents is also of utmost importance.

A 28 year old woman who is 38 weeks gestation is found to have a breech pregnancy. In order to ensure survival of the baby, she will need a C-section. Without one, there is a high likelihood that her fetus will not survive. When presented with the option to have a C-section, she replies, "There's no way you're cutting me open!" What is the most appropriate next step?

- A) Attempt to obtain consent for the C-section from the father of the baby
- B) Inform the patient that she cannot refuse life-saving measures for her unborn fetus, and perform the C-section
- C) Respect her wishes and allow her to refuse C-section

Answer:

C) Respect her wishes and allow her to refuse C-section

As long as the child is still in the uterus, the mother can accept or refuse whichever treatment she sees fit. She may refuse a C-section even if this will put the unborn fetus' life at risk. The father has no legal right to make a decision on any pregnancy-related issue. Only the mother can sign informed consent during pregnancy.

A 65 year old male suffers from chronic kidney disease and receives dialysis three times per week. He has consented to this treatment without objection. After six months, he informs you that he has now changed his mind and would rather die than continue undergoing dialysis. He is not depressed or suicidal. What is the most appropriate response?

- A) Stop his dialysis, knowing that he will likely suffer from volume overload and die
- B) Given that he has signed consent and will die without the treatment, continue his dialysis
- C) Stop his dialysis - if he returns in cardiac arrest, then emergently dialyze him
- D) Obtain a psychiatry consult
- E) Get a court order to treat him on the basis that he will die without it

Answer:

A) Stop his dialysis, knowing that he will likely suffer from volume overload and die.

Patients have a right to try a certain therapy for a while and then change their mind. So long as they do not appear depressed and are of sound mind, ethics committees and psychiatrists do not need to be involved.

A 50 year old male presents to your clinic for routine follow up. At the end of your visit you decide to counsel him on smoking cessation. He has smoked 1 pack per day for the last twenty years. Which of the following statements is most likely to be effective in encouraging the patient to stop smoking?

- A) "Smoking can cause many different types of cancer"
- B) "Why do you continue to smoke when you know how bad it is for your health?"
- C) "Have you ever thought about quitting smoking?"
- D) "Instead of spending all that money on cigarettes, you could be donating it to charity. Wouldn't that make you feel happier?"
- E) "From electric cigarettes to nicotine patches, there are a lot of options available to help you quit. Is there one in particular that you would like to try?"

Answer:

C) “Have you ever thought about quitting smoking?”

Asking an open ended question to gauge the level of a patient's interest in quitting is the best place to start. Attempting to scare a patient (whether it's by listing off all of the diseases associated with smoking or by showing pictures of smokers' lungs) is never a very effective method. Making the patient feel guilty about smoking is also not ideal as it does not help start a conversation about the patient's health, which is what should really be at the center of this. Simply naming the different options and asking the patient which one he wants also won't help accomplish your goal, as it does not address the issue of whether or not the patient is interested in quitting in the first place.

A patient that you have known and treated for years presents to your clinic. He informs you that he has cheated on his wife and is concerned that he may have contracted a sexually transmitted disease. Cultures are sent and return positive for gonorrhea. After treating the patient, you inform him of the need to tell his wife, who also happens to be your patient and well known to you. He tells you that he does not want to, as it would threaten his marriage. After encouraging him as best as you can to inform his wife, what is the appropriate response?

- A) Respect his right to confidentiality
- B) Notify the Department of Health
- C) Arrange for the wife to have an appointment in your clinic. Test her at that time, and if the results are positive, treat her appropriately.
- D) Decline to treat the patient unless he agrees to tell his wife
- E) Have your clinic nurse call the wife to deliver the husband's results

Medical Ethics 101

Answer:

B) Notify the Department of Health

It is legal to break confidentiality for certain situations, including partner notification for sexually transmitted diseases. The patient's right to confidentiality in such cases is outweighed by the partner's right to safety.

As you are leaving for work, your neighbor happens to be picking up his newspaper. He stops to say hello, then states, "Can I ask you a question? My two year old's come down with a fever. He seems to be acting fine. I gave him some tylenol and his temperature came down – is there anything else I need to do?" You didn't even realize he had a two year old, but reassure him that there are lots of viruses going around and as long as he looks okay and his temperature is controlled he should be fine. Later that night, your spouse tells you, "The neighbor called, irate, because they took junior to the hospital and he was diagnosed with bacterial meningitis. Oh, and he says he's going to sue you..." Are you legally responsible?

A) Yes

B) No

Answer:

B) No, but this should carry an asterisk

The existence of a physician-patient relationship is a prerequisite to any malpractice claim. Courts have consistently ruled that no physician-patient relationship exists within an informal consultation. In the absence of such a relationship, there is no grounds for malpractice.

At the same time, consulting with a patient outside the exam room in an informal setting in no way lessens your legal liability. The best course of action is to tell the patient to come to the office the next day or, if the situation seems serious, send the patient to the emergency room. Physicians should be cautious in informal conversations because some courts have ruled the conversation can be enough to create a physician-patient relationship in which you can be held liable for a bad outcome. Physicians often think that a physician-patient relationship cannot be established without some type of formality, but that isn't necessarily the case.

A 50 year old woman presents with left breast pain. On exam the breast is swollen, there is dimpling of the areola, and there is bloody discharge from the nipple. A presumptive diagnosis of breast cancer is made. The patient states that her sister was diagnosed with breast cancer and suffered through years of chemotherapy. She informs you that if test results confirm she has cancer, she would become depressed. The patient's biopsy comes back positive for invasive carcinoma. If surgically removed, the patient will have a significant chance of recovery. Which of the following is most appropriate?

- A) The patient is incompetent and therefore can neither consent nor refuse consent for surgery, so she should be taken to the operating room
- B) Do not inform the patient of her diagnosis, but take her to the operating room for surgery as she does not have capacity to make decisions
- C) Do not inform the patient of her results and attempt to contact family members
- D) Inform the patient of her results as she has a right to know, and then allow her to decide if she wants to have surgery or not
- E) Inform the patient of her results, then discuss all of the different methods of treatment and the risk/benefit of each
- F) Inform the patient that the results did not show cancer, but that you would need to remove the mass, and take her to the operating room

Answer:

E) Inform the patient of her results, then discuss all of the different methods of treatment and the risk/benefit of each

In this case there is no reason the patient should not be informed of her test results. Not only does she have the right to know, but she has the right to make a fully informed decision on how to proceed. Informing her without discussing other treatment options is inadequate.

Competency is a legal term referring to a right to determine one's own affairs after the age of eighteen. Adults are assumed to be competent until proven otherwise. To deem someone incompetent would require legal proceedings – lack of competence can never be determined by a physician and any answer choice that presumes you will determine if a patient is competent or not is automatically wrong.

Before allowing a patient to make a decision, you must ensure that he or she has capacity – they should be alert and oriented and possess the ability to assess the relative merits of different options in a reasonable fashion, comprehend relevant issues and potential outcomes, choose between different options, express a choice, and explain their reasons. Consistency is also important – in other words, if a patient has rapid fluctuations in their choice from moment to moment that should make you question their capacity. The presence of depression or dementia is not synonymous with lack of capacity.

Therapeutic privilege is an exception to informed consent - if giving the information would severely harm the patient or undermine the informed consent process (ie if the patient were to become suicidal upon finding out her results) – it might be appropriate to act without informing the patient of her diagnosis.

Your brother, who lives just a few miles away, calls you at 7am and informs you that he has run out of his albuterol and can't see his physician for another week. He asks if you can call in a refill for him. You have never seen him as a patient before but he gives you his pharmacy's phone number and asks that you do it as he is going to the lake in a few hours and thinks he will need it when he gets there. Can you legally refill?

A) Yes

B) No

Answer:

A) Yes, legally

This is a gray area and should be handled with care. The AMA Code of Medical Ethics states: “physicians generally should not treat themselves or members of their immediate families because their professional objectivity may be compromised in those situations.” There are numerous reasons your decision-making might be influenced:

- Personal feelings might skew your objectivity
- A desire to avoid necessary but potentially embarrassing questions
- The patient might be hesitant to discuss sensitive information which could be relevant (abdominal pain, pregnant? Chest pain, stress? Sleep disorders, depression?)
- You might over-reach and treat something outside of your scope

Rules can vary state to state, but most state medical boards discourage such prescribing of medications and, if necessary, encourage documentation. Two areas where it may be permissible are in cases of emergencies and in isolated settings where no other qualified physician is readily available. Physicians should not be the primary or regular care providers for their immediate family members, but giving routine care for short-term minor problems may be acceptable.

The same brother calls you again – he just came back from his trip and has an appointment with his doctor in two days. He picked up a cough while gone and has used tylenol #3 elixir in the past which helps. He asks you to write him enough for three doses – a total of 20mL and well below any amount that can cause overdose or significant side effects. He will come by and pick it up from your office. Can you legally prescribe?

A) Yes

B) No

Answer:

B) No

While rules governing the use of written prescriptions can vary from state to state, with controlled substances the federal government becomes involved. In such cases, a prescriber must have a patient-physician relationship, including a written record. Many states further require documenting a medical history and a physical exam before prescribing any controlled substance.

A 12 year old child is admitted to the hospital for further workup of extremity pain. His x-ray is concerning for osteosarcoma and a biopsy is done which confirms these findings. After privately informing the parents of the test results, you go into his room to see if his pain is well controlled. He asks if you have the results back and if he will need surgery or if he can go home. What is the most appropriate response?

- A) "We don't have the test results back"
- B) "We got those results back, but will need to run some additional tests before making a decision"
- C) "What have your parents told you?"
- D) "You have osteosarcoma, which is a type of cancer in your bone"
- E) "I think someone's paging me...I'll be right back"

Answer:

C) “What have your parents told you?”

So long as the patient is under the age of eighteen (and is not considered a 'mature minor'), only his or her parents/guardians can determine what information is given regarding test results and diagnoses. Ideally you would go into the room, together with the parents, and inform the child of his condition and what the next step in management will be. However, if for whatever reason the parents choose to not inform their child, you must follow suit.

A 35 year old female who has been your patient for several years comes to your clinic. She confides that she and her husband have been unsuccessfully trying to have children for several months now. You decide to refer her to an OB/Gyn to start an infertility workup. Her husband schedules an appointment to see you the next day. He confesses to having had a vasectomy one year before he met his wife, and has never told her this before. He is aware of everything you discussed with her on the previous day. What should you do?

- A) You have an obligation to inform the wife, as she is also your patient and you are leading her down a potentially expensive and time consuming process of having an unnecessary infertility workup
- B) You have no obligation to tell the wife under any circumstance
- C) You have no obligation to tell the wife unless her workup is negative, as a means to explain why she might not be able to have children
- D) You have an obligation to inform the wife, as this directly impacts the care you give and can be considered part of her 'social/family history'

Answer:

B) You have no obligation to tell the wife under any circumstance

The patient's wife might legitimately have an undiagnosed cause of infertility, and husband might actually be fertile – there is no reason to breach confidentiality; in fact, you have a duty to maintain it. There is no mandate or precedent that states one spouse must always be informed of medical care received by the other. Therefore you should do the testing, but encourage the patient to inform his wife.

A 25 year old woman brings her two month old baby into your office for routine vaccinations. However, on this visit the mother states that she has recently read some articles questioning the safety of mercury and thimerosal in vaccines, and a possible link with autism. She is unsure if the vaccines are safe for her baby. How should you respond?

- A) "There is no study that has shown a link between the mercury/thimerosal in vaccinations and autism or any other problems."
- B) "Let's check a mercury and thimerosal level in your child and if they are high we can discuss alternatives."
- C) "Can you bring in those articles on your next visit and we can go over them together?"
- D) "Some studies have suggested a link but there is no definitive evidence - the American Association of Pediatrics still recommends the vaccinations, and so do I."
- E) "While there are reported risks, if we don't administer the vaccines, your child could acquire measles, polio, or hepatitis, amongst others – all of which could be deadly."
- F) "I give these to every child – I haven't had problems with any of them yet."

Answer:

D) “Some studies have suggested a link but there is no definitive evidence - the American Association of Pediatrics still recommends the vaccinations, and so do I.”

It is important to acknowledge the mother's viewpoint while at the same time giving sound medical advice. While there have been reported links, all major medical societies still recommend the immunization schedule sponsored by the American Association of Pediatrics (AAP). There have been alternative vaccination schedules that have been proposed, in which the duration between different vaccines is increased so they are spread out over a longer period of time, but none have officially received endorsement. Threatening a mother with all the various diseases that vaccinations offer protection from is not the most effective way to get her to comply. Having her bring in her own articles unnecessarily delays administration of vaccines – additionally, there are studies perporting such a link and going over those with her is not the best use of time.

Thimerosal, previously used as a preservative in many recommended childhood vaccines, was removed/reduced to trace amounts in 2001. There has been no recent decrease in autism despite the exclusion of anything more than trace levels of thimerosal from nearly all childhood vaccines – leading many to conclude that data does not support the hypothesis that exposure to thimerosal during childhood is a primary cause of autism.

A patient presents to your clinic with wrist pain. X-rays are nondiagnostic; eventually you obtain an EMG which confirms a diagnosis of carpal tunnel syndrome. He will need a referral to an orthopedic surgeon for definitive care. There is a new orthopedic surgeon in town who is looking for new patients. You have met him before and he seemed genuine although you are unsure of the specifics of where he trained. He is offering \$20 for each referral that you send to him. What is the most appropriate next step?

- A) Refer the patient to the new physician
- B) Refer the patient to the new physician but decline the \$20 referral fee
- C) Refer the patient to the new physician but give the \$20 referral fee to the patient
- D) Refer the patient to a different group of orthopedic surgeons, one that your partners traditionally use
- E) Refer the patient to the new physician and accept the referral fee, but inform the patient so that he is aware of it

Answer:

D) Refer the patient to a different group of orthopedic surgeons, one that your partners traditionally use

Accepting payment for a referral is illegal and should never be tolerated. Referral fees are payments that medical service providers give to doctors in exchange for the physician referring patients to that service. Such referral fees are both ethically and legally frowned upon. The American Medical Association's Code of Medical Ethics requires doctors to provide patients with "relevant information" about potential procedures, but has no guidelines on what to tell them about the specialist to whom they're being sent – however it violates the basic trust that is at the center of a physician-patient relationship.

KEY ETHICAL PRINCIPLES:

Autonomy	Respecting a patient's right to make their own decision regarding medical care
Beneficence	Actions done for the benefit of others (CPR, vaccinations, education and counseling)
Non-maleficence	Do no harm (Stopping harmful medications, not prescribing treatments that are ineffective)
Justice	Be as fair as possible when offering treatments to patients and allocating scarce medical resources

Double Effect: Two different types of consequences from one action; usually combined beneficence and non-maleficence (morphine as both an analgesic and a respiratory depressant in a dying patient)

Patients have a right to autonomy even if the decision is unwise; however beneficence dictates that a physician act in the patient's best interest. Autonomy is only possible when a patient possesses the ability make decisions; the following are required components:

- Understand the medical diagnosis and prognosis
- Understand the recommendations and alternatives
- Acknowledge the risk and benefit of each alternative
- Using logical reasoning to make a decision
- Decision does not fluctuate and remains stable with time
- Lack of delusions/hallucinations

Capacity is determined by physicians; competency by courts and judges

Paternalism: Healthcare professionals make decisions for the patient

Withholding information when having that information can cause serious psychological harm to a patient

Informed Consent components:

- Full disclosure
- Comprehension on behalf of the patient (capacity)
- Voluntariness (lack of coercion)

Consent: Presumed consent obtained when a patient is either unconscious or incompetent or no surrogate decision maker is present

Good Samaritan requirements:

- True emergency (potential for loss of life or limb)
- No compensation (care is given in good faith)
- No abandonment (once care is commenced you must remain at the scene until someone capable of taking over arrives)
- No gross negligence

components; all four are required for a successful claim:

- **Duty of care**
- **Deviation from standard of care**
- **Damage to the patient**
- **Damage occurred directly as a result of physician's actions**

“In the end, every ethical rule must be tested against real life stories”

- Stephen Gillers, *Yale Law Journal*

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USMLE™ Step 1 prep!

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